5660 W Flamingo RdSTE A
Las Vegas NV 89103
Phone: (702)416-0611 Fax: (702)899-4479
NPI: 1720712235

STAFF INFO
Name: Garcia Alfonso, Kelvyn RN
Patient INFO
Name: HYLTON, ALICE
DOB: 1964-01-19

OASIS Assessment: OASIS E1 - START OF CARE

ADMINISTRATIVE INFORMATION

(M0010) Agency Medicare Provider Number
297328

(M0014) Branch State
Branch State **Nevada**

(M0016) Branch ID Number

(M0018) Attending Physician (NPI)
1275380313

Physician
LEINHAUSER, JULIE

Adddress
2801 N TENAYA WAY STE C LAS VEGAS NV 89128

- [ ] Not Applicable

(M0020) Patient ID number
120581

(M0030) Start of Care Date
02/18/2026

(M0032) Resumption of Care Date
mm/dd/yyyy

- [x] Not Applicable

(M0040) Patient Name

First Name	Middle Name	Last Name	Suffix
ALICE		HYLTON	

(M0050) Patient State of Residence
State of Residence **Nevada**

(M0060) Patient Zip Code
89108

(M0063) Medicare Number
4UW7GF9VQ40

- [ ] Not Applicable

(M0064) Social Security Number

- [x] Not Applicable

(M0065) Medicaid Number

- [x] Not Applicable

(M0066) Birth Date *
01/19/1964

(M0069) Gender *
Gender **Female**

(A1005) Ethnicity Are you of Hispanic, Latino/a, or Spanish origin? Check all that apply

- [x] A. No, not of Hispanic, Latino/a, or Spanish origin
- [ ] B. Yes, Mexican, Mexican American, Chicano/a
- [ ] C. Yes, Puerto Rican
- [ ] D. Yes, Cuban
- [ ] E. Yes, Another Hispanic, Latino, or Spanish origin
- [ ] X. Patient unable to respond
- [ ] Y. Patient declines to respond

(A1010) Race What is your race?

- [ ] A. White
- [x] B. Black or African American
- [ ] C. American Indian or Alaska Native
- [ ] D. Asian Indian
- [ ] E. Chinese
- [ ] F. Filipino
- [ ] G. Japanese
- [ ] H. Korean
- [ ] I. Vietnamese
- [ ] J. Other Asian
- [ ] K. Native Hawaiian
- [ ] L. Guamanian or Chamorro
- [ ] M. Samoan
- [ ] N. Other Pacific Islander
- [ ] X. Patient unable to respond
- [ ] Y. Patient declines to respond
- [ ] None of the above

(M0150) Current Payment Sources for Home Care: (Mark all that apply.) *

- [ ] None; no charge for current services
- [x] Medicare (Traditional fee-for-service)
- [ ] Medicare (HMO/managed care/Advantage plan)
- [ ] Medicaid (Traditional fee-for-service)
- [ ] Medicaid (HMO/managed care)
- [ ] Worker's compensation
- [ ] Title programs (for example, Title III, V, or XX)
- [ ] Other government (for example, TriCare, VA)
- [ ] Private insurance
- [ ] Private HMO\/managed care
- [ ] Self-pay
- [ ] Other (specify)
- [ ] Unknown

A1110. Language

A.What is your preferred language?
English.

B. Do you need or want an interpreter to communicate with a doctor or health care staff?

- (●) 0 - No
- ( ) 1 - Yes
- ( ) 9 - Unable to determine

(M0080) Discipline Completing Assessment

- (●) RN
- ( ) PT
- ( ) SLP/ST
- ( ) OT

Assigned Staff
Garcia Alfonso, Kelvyn RN

(M0090) Date Assessment Completed:
02/18/2026

(M0100) Assessment Reason

- [x] Start of Care

(M0102) Date of Physician-ordered Start of Care (Resumption of Care)

If the physician indicated a specific start of care (resumption of care) date when the patient was referred for home health services, record the date specified.
Skip to A1250, Transportation, if date entered

mm/dd/yyyy

(M0104) Date of Referral:

Indicate the date that the written or verbal referral for initiation or resumption of care was received by the HHA

02/13/2026

- [x] NA - No specific SOC/ROC date ordered by physician

(A1250) Transportation Has lack of transportation kept you from medical appointments, meetings, work, or from getting things needed for daily living?

- [x] A - Yes, it has kept me from medical appointments or from getting my medications
- [ ] B - Yes, it has kept me from non-medical meetings, appointments, work, or from getting things that I need
- [ ] C - No
- [ ] X - Patient unable to respond
- [ ] Y - Patient declines to respond

(M1000) Inpatient Facilities that patient was discharged within the past 14 days? (Mark all that apply.)

- [ ] 1 - Long-term nursing facility (NF)
- [ ] 2 - Skilled nursing facility (SNF/TCU)
- [ ] 3 - Short-stay acute hospital (IPPS)
- [ ] 4 - Long-term care hospital (LTCH)
- [ ] 5 - Inpatient rehabilitation hospital or unit (IRF)
- [ ] 6 - Psychiatric hospital or unit
- [ ] 7 - Other (specify)
- [x] NA - Patient was not discharged from an inpatient facility _ **[Go to M1021]**

(M1005) Inpatient Discharge Date (most recent)

mm/dd/yyyy

- [ ] UK - Unknown

Primary Reason for Home Health / Medical Necessity

The patient is considered homebound because she has multiple active medical conditions, including polyneuropathy, muscle weakness, bilateral lower leg edema, left foot drop, pressure injury of the buttock, memory impairment, and dependence on an enabling machine or device. These conditions significantly impair her mobility and ability to perform activities of daily living independently, resulting in difficulty walking, dressing, bathing, grooming, toileting, and doing errands alone. She is unable to walk independently without assistance or assistive devices and has transportation difficulties. Additionally, she experiences tremors, chronic pain, anxiety, depressive disorder, and seizures, which further complicate her functional status and safety.

Leaving the home requires a considerable and taxing effort due to her severe mobility impairments, muscle weakness, and neurological deficits such as foot drop and neuropathy. She also has a history of falls and requires maximal assistance, which increases the risk of injury if she attempts to leave the home unassisted. Any outings are limited to medically necessary appointments or essential activities, given her dependence on assistive devices and high fall risk.

These documented functional limitations and medical conditions justify the patient's homebound status and the medical necessity for skilled home health services to provide ongoing skilled nursing, physical and occupational therapy, and assistance with complex medical management and safety oversight.

Homebound Reason

- [ ] Document as Clinical Narrative
- [ ] Patient is not Homebound

Criteria One (One must be met): *Describe what assistive devices the patient is dependent on (if any) and specifically why the patient condition warrants the special transportation or the assistance of another person or describe the condition that makes leaving home medically contraindicated.*

Criteria 1 A: Because of illness or injury

- [x] Patient needs the aid of supportive devices
  - [ ] Crutches
  - [ ] Cane
  - [ ] Manual Wheel Chair
  - [ ] Walker
  - [x] Motorized Wheelchair
  - [x] Scooter
- [x] Patient Needs the use of special transportation.
- [x] Patient needs the assistance of another person to leave the place of residence

Criteria 1 B: Has a condition such as leaving his or her home is medically contraindicated.

- [ ] Status post surgery
- [ ] Angina upon minimal exertion
- [ ] Immuno deficiency

Other Homebound Reason :

Criteria Two (Both must be met)

Criteria 2 A: There must be exist a normal inability to leave home.

- [ ] Dyspnea on minimal exertion
- [ ] Bedbound
- [x] Chair bound
- [ ] Dementia
- [ ] Blindness
- [ ] Angina upon minimal exertion
- [ ] Paralysis
  - [ ] Quadprilegic or
  - [ ] Paraplegic or
  - [ ] Hemiplegia
- [x] Unable to tolerate standing or walking for extended periods
- [x] Experiencing pain impairs/restricts mobility
- [x] Balance impaired with potential for falls; uses walls and furniture to ambulate
- [x] Leaving home is exhausting, extended rest periods to recover
- [ ] Extended respiratory recovery after exertion
- [ ] Weakness and pain from recent surgery
- [ ] Visually impaired-requires constant attendance/assist
- [ ] Cognitively impaired, unable to safely leave home without assist
- [ ] Infectious disease process causing weakness and exhaustion
- [x] Narcotics impaired ability to safely leave home unassisted

- [x] Requires human assist due to unsteady gait/poor balance
- [ ] Bedbound and requires EMS support

Other Homebound Reason :
Weakness from a progressive condition.

Criteria 2 B: Leaving home must require a considerable and taxing effort

- [x] Special transportation
- [x] Severe pain on ambulation
- [x] Leaving familiar environment causes fear and confusion
- [x] Leaving home is strenous requiring extended rest to recover
- [ ] Fatigue to arrange/use appropriate transportation
- [x] Requires assistive device to ambulate
- [ ] Exhausting to navigate vehicle and require human assist
- [x] Requires assist to manage equipment and public transportation
- [x] Needs assistance to handle and transport equipment
- [x] Activities restricted due to pain limitations

Other Homebound Reason :

Prognosis

○ 1-Poor ● 2-Guarded ○ 3-Fair ○ 4-Good ○ 5-Excellent

Functional Limitations

○ 1 - Amputation
○ 2 - Bowel/Bladder (Incontinence)
○ 3 - Contracture
○ B- Other (Specify)
○ 4 - Hearing
○ 5 - Paralysis
● 6 - Endurance
● 7 - Ambulation
○ 8 - Speech
○ 9 - Legally blind
○ A - Dyspnea with minimal exertion

Emergency Preparedness

Obtained the patient's emergency contact numbers for the medical record
Discussed the HHA's plans for supporting their patients during a natural or man-made disaster
Discussed patient specific emergency planning options
Discussed the development of the patient's individualized emergency preparedness plan of care, including self-care readiness and the procedure to follow up with the HHA in the event services are interrupted
Notified local utility companies and local emergency offices notified of life supporting equipment being used
State and local emergency preparedness officials notified about the possible need for evacuation
List of recommended items to have prepared/ready and available in the event of an emergency
Educational materials provided to suggest/assist with emergency management/decision making priorities
List of local and state approved evacuation routes and community shelters relevant to the patient's specific geographic location
Written materials to restate/reinforce the emergency preparedness procedures given to the Patient, and Representative.

Acuity Level
Medium Risk or Medium Priority

Emergency Triage
Category 3 - Visit could be postponed 24-48 hours without adverse effects

Additional Emergency Preparedness Information

- [x] Needs assistance during an emergency
- [x] Contact made with local/state emergency preparedness officials regarding patient in need of help during an evacuation
- [x] Medical Needs/Equipment (i.e. bedbound, oxygen, vent, IV cardiac meds other DME)

Comments

Advance Directives

Patient has Advance Directives? ● Yes ○ No
If you have checked "yes", fill the following fields:
Advance Directives (select the entities that apply to you): ● Do Not Resuscitate (DNR) ○ Living Will
- [ ] Medical Power of Attorney

Name **Phone**

Other
Patient verbalized that is partially full coded declining mechanic ventilation and/or artificial life.

Copies on file at agency? ○ Yes ● No
Has surrogate? ● Yes ○ No

Name Jayson Hylton
Phone 951-380-0797

Patient was provided verbal and written information on Advance Directives? ● Yes ○ No
Is the Patient DNR (Do not Resuscitate)? ● Yes ○ No

Cultural

Primary Language?
- [x] English
- [ ] Spanish
- [ ] Chinese
- [ ] Vietnamese

Other

Is religion important to the patient? ● Yes ○ No
Patient's religious preference:
Patient's primary source of emotional support:

VITAL SIGNS

(M1060) Height and Weight - While measuring, if the number is X.1-X.4 round down; X.5 or greater round up

a. Height (in inches). Record most recent height measure since the most recent SOC/ROC
62

b. Weight (in pounds). Base weight on most recent measure in last 30 days; measure weight consistently, according to standard agency practice (for example, in a.m. after voiding, before meal, with shoes off, etc.
132

BMI
24.14

Comments

Vital Signs

Temperature: 99 Temporal
Respiratory (per minute): 22
Blood Pressure mm/Hg (R): 152 / 92 lying
Blood Sugar (mg/dl): Non-Fasting
O2 Sat: 92

Pulse (per minute): Apical = Radial = 100
Weight: 132 reported
Blood Pressure mm/Hg (L): /

O2 Sat in: Room Air

NEUROLOGICAL (HEAD)

(B1300) Health Literacy

How often do you need to have someone help you when you read instructions, pamphlets, or other written material from your doctor or pharmacy?

- ◯ **0. Never**
- ◯ **1. Rarely**
- ● **2. Sometimes**
- ◯ **3. Often**
- ◯ **4. Always**
- ◯ **7. Patient declines to respond**
- ◯ **8. Patient unable to respond**

(C0100) Should Brief Interview for Mental Status (C0200-C0500) be Conducted?

Attempt to conduct interview with all patients. ◯ **No (patient is rarely/never understood)** ● **Yes** ◯ **Not assessed/no information**

Brief Interview for Mental Status (BIMS)

(C0200) Repetition of Three Words

Ask patient: **"I am going to say three words for you to remember. Please repeat the words after I have said all three. The words are: SOCK , BLUE , and BED . Now tell me the three words."**

Number of words repeated after first attempt ◯ **None** ◯ **One** ◯ **Two** ● **Three** ◯ **Not Assessed**

After the patient's first attempt, repeat the words using cues (**"sock, something to wear; blue, a color; bed, a piece of furniture"**). You may repeat the words up to two more times.

(C0300) Temporal Orientation (Orientation to year, month, and day)

Ask patient: **"Please tell me what year it is right now?"**

A) Able to report correct year

- ◯ **0. Missed by > 5 years or no answer**
- ◯ **1. Missed by 2-5 years**
- ◯ **2. Missed by 1 year**
- ● **3. Correct**
- ◯ **Not assessed/no information**

Ask patient: **"What month are we in right now?"**

B) Able to report correct month

- ◯ **0. Missed by > 1 month or no answer**
- ◯ **1. Missed by 6 days to 1 month**
- ● **2. Accurate within 5 days**
- ◯ **Not assessed/no information**

Ask patient: **"What day of the week is today?"**

C) Able to report correct day of the week

- ● **0. Incorrect or no answer**
- ◯ **1. Correct**
- ◯ **Not assessed/no information**

(C0400). Recall

Ask patient: **"Let's go back to an earlier question. What were those three words that I asked you to repeat?" If unable to remember a word, give cue (something to wear; a color; a piece of furniture) for that word."**

A) Able to recall "sock"

- ◯ **0. No – could not recall**
- ● **1. Yes, after cueing ("something to wear")**
- ◯ **2. Yes, no cue required**
- ◯ **Not assessed/no information**

B) Able to recall "blue"

- ◯ **0. No – could not recall**
- ◯ **1. Yes, after cueing ("a color")**
- ● **2. Yes, no cue required**
- ◯ **Not assessed/no information**

C) Able to recall "bed"

- ◯ **0. No – could not recall**
- ● **1. Yes, after cueing ("a piece of furniture")**
- ◯ **2. Yes, no cue required**
- ◯ **Not assessed/no information**

C0500. BIMS Summary Score

Add scores for questions C0200-C0400 and fill in total score (00-15) **Enter 99 if the patient was unable to complete the interview** **12**

(C1310) Signs and Symptoms of Delirium (from CAM©)

A. Acute Onset of Mental Status Change

Is there evidence of an acute change in mental status from the patient's baseline?

● **0. No** ◯ **1. Yes**

B. Inattention

Did the patient have difficulty focusing attention, for example, being easily distractible or having difficulty keeping track of what was being said?

- ● **(0) Behavior not present**
- ◯ **(1) Behavior continuously present, does not fluctuate**
- ◯ **(2) Behavior present, fluctuates (comes and goes, changes in severity)**
- ◯ **Not assessed/no information**

C. Disorganized thinking

Was the patient's thinking disorganized or incoherent (rambling or irrelevant conversation, unclear or illogical flow of ideas, or unpredictable switching from subject to subject)?

- ● **(0) Behavior not present**
- ◯ **(1) Behavior continuously present, does not fluctuate**
- ◯ **(2) Behavior present, fluctuates (comes and goes, changes in severity)**
- ◯ **Not assessed/no information**

D. Altered level of consciousness

Did the patient have altered level of consciousness, as indicated by any of the following criteria?

- vigilant – startled easily to any sound or touch □ lethargic – repeatedly dozed off when being asked questions, but responded to voice or touch
- stuporous – very difficult to arouse and keep aroused for the interview
- comatose – could not be arouse

- ● **(0) Behavior not present**
- ◯ **(1) Behavior continuously present, does not fluctuate**
- ◯ **(2) Behavior present, fluctuates (comes and goes, changes in severity)**
- ◯ **Not assessed/no information**

(M1700) Cognitive Functioning

Patient's current (day of assessment) level of alertness, orientation, comprehension, concentration, and immediate memory for simple commands

- ● **Alert/oriented, able to focus and shift attention, comprehends and recalls task directions independently.**
- ◯ **Requires prompting (cuing, repetition, reminders) only under stressful or unfamiliar conditions.**
- ◯ **Requires assistance and some direction in specific situations (for example, on all tasks involving shifting of attention) or consistently requires low stimulus environment due to distractibility**
- ◯ **Requires considerable assistance in routine situations. Is not alert and oriented or is unable to shift attention and recall directions more than half the time.**
- ◯ **Totally dependent due to disturbances such as constant disorientation, coma, persistent vegetative state, or delirium.**

(M1710) When Confused (Reported or Observed Within the Last 14 Days):

- ● **Never**
- ◯ **In new or complex situations only**
- ◯ **On awakening or at night only**
- ◯ **During the day and evening, but not constantly**
- ◯ **Constantly**
- ◯ **Patient nonresponsive**

(M1720) When Anxious (Reported or Observed Within the Last 14 Days):

- ◯ **None of the time**
- ◯ **Less often than daily**
- ● **Daily, but not constantly**
- ◯ **All of the time**
- ◯ **Patient nonresponsive**

Neuro/Emotional/Behavioral Assessment

Conditions of Participation

Psychosocial

- ☐ **WNL (Within Normal Limits)**
- ☐ **Poor Home Environment**
- ☐ **Impaired Decision Making**
- ☐ **Poor Coping Skills**
- ☐ **Agitated**
- ☐ **Inappropriate Behavior**
- ☑ **Depressed Mood**

- [x] Demonstrated/Expressed Anxiety
- [ ] Document as Clinical Narrative
- [ ] Irritability

Educational level

High school.

Primary Language

English.

Unable to: ☐ *Read* ☐ *Write*

Barriers:

Language

- ◯ Needs interpreter

Learning

- ◯ Mental
- ◯ Physical
- ◯ Psychosocial
- ◯ Functional

Emotions Reported:

- ◯ Angry
- ◯ Discouraged
- ◯ Withdrawn
- ◯ Flat affect
- ◯ Difficulty coping
- ◯ Disorganized

- ● Depressed
- ◯ Recent
- ◯ Long Term

Treatment:

Evidence of abuse/neglect/exploitation:

- ◯ Potential
- ◯ Verbal/Emotional
- ◯ Physical
- ◯ Actual
- ◯ Financial
- ● No Evidence of Abuse/Neglect/Exploitation

Unable to cope with altered health status, patient:

- ● Lacks motivation
- ◯ Has unrealistic expectations
- ◯ Denies problems
- ◯ Unable to recognize problems

Sleep/Rest

- ● Adequate
- ◯ Inadequate

No. of hours slept per night: **7**

Comments

Coping skills affected by lacks motivation due to the progressive condition presented.

Spiritual / Cultural implications that impact care

Spiritual resource

☐ Inappropriate responses to caregivers/clinician

☐ Inappropriate follow-through in past

Did patient drive a vehicle before admission? ☐Yes ☑No ☐NA

Did the patient have a job before admission? ◯Yes ◉No

If yes, do they want to drive post-discharge? ☐Yes ☐No

If yes, do they want to return to work post-discharge? ◯Yes ◯No

Mental Status:

- ● 1 - Oriented
- ● 4 - Depressed
- ◯ 7 - Agitated
- ◯ 10 - Dementia
- ◯ 13 - Wanders
- ◯ 2 - Comatose
- ◯ 5 - Disoriented
- ● 8 - Alert
- ◯ 11 - Assaultive
- 14 - Other

Anxious.

- ◯ 3 - Forgetful
- ◯ 6 - Lethargic
- ◯ 9 - Confused
- ◯ 12 - Delirious

Describe the patient's mental status. Include general appearance, behaviors, emotional responses, mental functioning, overall social interaction as well as both clinical objective observations and subjective descriptions reported during this visit.

Has there been a sudden/acute change in their mental status? ☐ Yes ☑ No Reported by: ☐ If yes, did the change coincide with something else like medication change, fall, or loss of a loved one? ☑ No ☐ Yes

Comments:

Neuromuscular

Impairment:

- [x] No Problems identified

- [ ] Speech
- [ ] Hearing
- [ ] Visual
- [ ] Tremors
- [ ] Numbness
- [ ] Tingling
- [ ] Vertigo
- [ ] Ataxia
- [ ] Decreased Sensitivity

Fall(s) since last visit (Specify):

Grip Strength:

- [ ] Normal
- [ ] Strong
- [x] Weak
- [x] Equal
- [ ] Unequal
- [ ] Bilateral
- [ ] Right
- [ ] Left

Others (Specify):

Grasp Strength:

- [ ] Normal
- [ ] Strong
- [x] Weak
- [x] Equal
- [ ] Unequal

Mobility:

- [ ] Balance Within Normal Limits
- [x] Unsteady Gait

Weakness (Specify):

Generalized.

Change in ADL (Explain):
Dependent.

EYES AND EARS

B0200. Hearing

Ability to hear (with hearing aid or hearing appliances if normally used)

- [x] **0. Adequate – no difficulty in normal conversation, social interaction, listening to TV**
- [ ] **1. Minimal difficulty – difficulty in some environments (e.g., when person speaks softly, or setting is noisy)**
- [ ] **2. Moderate difficulty – speaker has to increase volume and speak distinctly**
- [ ] **3. Highly impaired – absence of useful hearing**

B1000. Vision

Ability to see in adequate light (with glasses or other visual appliances)

- [ ] **0. Adequate – sees fine detail, such as regular print in newspapers/books**
- [x] **1.Impaired – sees large print, but not regular print in newspapers/books**
- [ ] **2.Moderate impaired – limited vision; not able to see newspaper headlines but can identify objects**
- [ ] **3.Highly impaired – object identification in question, but eyes appear to follow objects**
- [ ] **4.Severely impaired – no vision or sees only light, colors or shapes; eyes do not appear to follow objects**

Pupils

- [ ] **No Problems Identified**
- [x] **PERRLA**
- [ ] **Anisocoria**
- [ ] **Right**
- [ ] **Left**
- [ ] **Others(Specify):**

Vision

- [ ] **No Problems Identified**
- [x] **Glasses**
- [ ] **Macular Degeneration**
- [ ] **Blurred Vision**
- [ ] **Glaucoma**
- [ ] **Contact Lenses:**
- [ ] **Right**
- [ ] **Left**
- [ ] **Cataracts**
- [ ] **Blindness(Specify):**
- [ ] **Others (Specify):**

Hearing

- [x] **No Problems Identified**
- [ ] **Right Hearing Impairment**
- [ ] **Left Hearing Impairment**
- [ ] **Deaf**
- [ ] **Speech Impairment (Specify):**

CARDIOVASCULAR

Heart Sounds

- [x] **Normal**
- [x] **Regular**
- [ ] **Irregular**
- [ ] **Murmur**
- [ ] **Gallops**
- [ ] **Clicks**
- [ ] **Friction Rule**

Comments (If Abnormal (Explain)):

Edema

Upper:

- [ ] **Right Extremities**
- [ ] **Left Extremities**

Lower:

- [ ] **Right Extremities**
- [ ] **Left Extremities**

Others:

Comments:

.

- [ ] **Cramps**
- [ ] **Claudication**
- [ ] **Fatigue**

Capillary Refill:

- [ ] **> 3 secs**
- [x] **< 3 secs**

PT/INR:

/

Peripheral Pulses

Radial:	[x] **Strong/Bounding**	[ ] **Weak**	[ ] **Thready**	[ ] **Absent**
Pedal:	[x] **Strong/Bounding**	[ ] **Weak**	[ ] **Thready**	[ ] **Absent**

Chest Pain

- [x] **Denies**
- [ ] **Anginal**
- [ ] **Postural**
- [ ] **Localized**
- [ ] **Substernal**
- [ ] **Sharp**
- [ ] **Dull**
- [ ] **Aching**
- [ ] **Radiating**
- [ ] **Viselike**
- [ ] **Stabbing**
- [ ] **Others (Specify):**

Associated with:

- [ ] **Shortness of Breath**
- [ ] **Activity**
- [ ] **Rest**
- [ ] **Frequency/Duration:**

Comments:

RESPIRATORY (CHEST/THORAX)

M1400. When is the patient dyspneic or noticeably Short of Breath?

- [ ] **0. Patient is not short of breath**
- [x] **1. When walking more than 20 feet, climbing stairs**
- [ ] **2. With moderate exertion (for example, while dressing, using commode or bedpan, walking distances less than 20 feet)**
- [ ] **3. With minimal exertion (for example, while eating, talking, or performing other ADLs) or with agitation**
- [ ] **4. At rest (during day or night)**

Respiratory Status

- [ ] **Dyspnea:**
- [ ] **At Rest**
- [ ] **With Exertion/Activity**
- [ ] **Accessory Muscle Used**
- [ ] **Orthopnea**
- [ ] **Retraction**
- [ ] **Stridor**

O2 **lpm** **O2 Saturation** **%**

- [ ] **PRN**
- [ ] **Continuous**
- [ ] **Mechanical Ventilator:**
- [ ] **Intubated**
- [ ] **Tracheostomy**

Others (Specify):

Lung Sounds

Anterior: Right: **Clear** Left: **Clear**

Posterior: Upper Right: **Clear** Upper Left: **Clear** Lower Right: **Clear** Lower Left: **Clear**

Cough

- [x] None
- [ ] Dry
- [ ] Acute
- [ ] Chronic
- [ ] Non-Productive
- [ ] Productive: [ ] Not Assessed/No Information [ ] Yes [ ] No
- Suction: [ ] Not Assessed/No Information [ ] Yes [ ] No
- [ ] Hemoptysis Frequency: Amount:

GASTROINTESTINAL (MOUTH TO ANUS)

K0520. Nutritional Approaches

	On Admission (Check All That Apply)
A. Parenteral/IV feeding	[ ]
B. Feeding tube (e.g., nasogastric or abdominal (PEG))	[ ]
C. Mechanically altered diet – require change in texture of food or liquids (e.g., pureed food, thickened liquids)	[ ]
D. Therapeutic diet (e.g., low salt, diabetic, low cholesterol)	[x]
Z. None of the above	[ ]

(M1870) Feeding or Eating

Current ability to feed self meals and snacks safely. Note: This refers only to the process of ***eating***, ***chewing***, and ***swallowing***, not preparing the food to be eaten.

- ( ) Able to independently feed self.
- ( ) Able to feed self independently but requires: (a) meal set-up; OR (b) intermittent assistance or supervision from another person; OR (c) a liquid, pureed or ground meat diet.
- (x) Unable to feed self and must be assisted or supervised throughout the meal/snack.
- ( ) Able to take in nutrients orally and receives supplemental nutrients though a nasogastric tube or gastrostomy.
- ( ) Unable to take in nutrients orally and is fed nutrients through a nasogastric tube or gastrostomy.
- ( ) Unable to take in nutrients orally or by tube feeding.

(M1620) Bowel Incontinence Frequency

- (x) Very rarely or never has bowel incontinence
- ( ) Less than once weekly
- ( ) One to three times weekly
- ( ) Four to six times weekly
- ( ) On a daily basis
- ( ) More often than once daily
- ( ) Patient has ostomy for bowel elimination
- ( ) Unknown

(M1630) Ostomy for Bowel Elimination

Does this patient have an ostomy for bowel elimination that (within the last 14 days): a) was related to an inpatient facility stay; or b) necessitated a change in medical or treatment regimen?

- (x) Patient does not have an ostomy for bowel elimination
- ( ) The ostomy was related to an inpatient stay or did necessitate change in medical or treatment regimen.
- ( ) Patient's ostomy was not related to an inpatient stay and did not necessitate change in medical or treatment regimen.

Mouth and Throat

- [x] No Problems Identified
- [ ] Dentures: [ ] Upper [ ] Lower [ ] Partial
- [ ] Bleeding Gums/Gingivitis [ ] Toothache [ ] Ulcerations [ ] Lesion
- [ ] Dysphagia [ ] Hoarseness [ ] Sore Throat [ ] Masses/Tumors

Comments:

Appetite

- [x] Good
- [ ] Fair
- [ ] Poor
- Intake: [ ] Adequate [ ] Inadequate

Abnormalities:

- [ ] Anoxeria
- [ ] Cachexia
- [ ] Nausea
- [ ] Vomiting
- [ ] Decreased Appetite
- [ ] Reflux/Indigestion
- [ ] Diarrhea
- [ ] Constipation
- [ ] Bowel Incontinence
- [ ] Other (Specify):

Feeding

- [ ] Formula
- [ ] Continuous Feeding cc hours
- [ ] Bolus:
- [ ] Gravity
- [ ] Pump cc every hour(s)

Flushing Protocol:

PEG/G-Tube Placement:

- [ ] Placement Checked
- [ ] Residual Checked Amount:

#16 Nutritional Requirements

- [ ] NPO
- [ ] No Added Salt
- [x] Low Salt
- [x] Low Fat
- [ ] Low Cholesterol
- [ ] No Concentrated Sweets
- [ ] Controlled Carbohydrates
- [ ] Regular
- [ ] Fluid Restriction:

Other:

Nutritional Health Screen	Score	Nutritional Health Screen	Score
( ) Without reason, has lost more than 10 lbs in the last 3 months	15	( ) Does not always have enough money to buy foods need	10
(x) Has an illness or condition that made pt change the type and/or amount of food eaten	10	( ) Eats alone most of the time	5
( ) Has open decubitus, ulcer, burn or wound	10		
( ) Eats fewer than 2 meals a day	10	(x) Takes 3 or more prescribed or OTC medications a day	5
		Total	15

Nutritional Health Screen	Score	Nutritional Health Screen	Score
◯ Has a tooth/mouth problem that makes it hard to eat	10	◯ Is not always physically able to cook and/or feed self and has no caregiver to assist	5
◯ Has 3 or more drinks of beer, liquor or wine almost every day	10	◯ Frequently has a diarrhea or constipation	5
		Total	15

◉ Good Nutritional Status (Score 0 - 25) ◯ Moderate Nutritional Status (Score 25 - 54) ◯ High Nutritional Status (Score 55 - 100)

Nutritional Status Commments:

Weight

☐ No Change ☑ Weight Gain ☐ Weight Loss

Abdomen

☑ Abdominal Soft/Tender ☐ Abdominal Distention ☐ Girth(cm)

Ostomy/Stoma Appearance

☐ Pinkished-Red ☐ Moist Abnormalities: ☐ Purple ☐ Black in Color ☐ Dry

☐ Gray ☐ Bad Odor ☐ Itches/Hurt Other (Specify):

Bowel Sounds:

Abdominal Quadrants:

☑ Right Lower Normoactive ☑ Right Upper Normoactive ☑ Left Lower Normoactive ☑ Left Upper Normoactive

Last Bowel Movement: 02/17/2026

Constipation

☐ Chronic ☐ Acute ☑ Occasional ☐ Internal Hemorrhoids

☐ External Hemorrhoids ☐ Laxative/Enema Use

Stool Appearance

☑ Within Normal Limits ☐ Gray ☐ Tarry ☐ Black ☐ Fresh Blood

☐ Others (Specify):

GENITOURINARY (GENITALS AND URINARY)

(M1600) Has this patient been treated for a Urinary Tract Infection in the past 14 days?

◉ No ◯ Yes ◯ Patient on prophylactic treatment ◯ Unknown

(M1610) Urinary Incontinence or Urinary Catheter Presence:

◯ Patient is incontinent
- ◯ Day time
- ◯ Night time
- ◯ Day and Night
- ◯ Timed-voiding defers incontinence
- ◯ Occasional stress incontinence

◯ Patient requires a urinary catheter (specifically: external, indwelling, intermittent, or suprapubic)

◉ No incontinence or catheter (includes anuria or ostomy for urinary drainage)

Genitourinary Assessment

☑ Within Normal Limits

Abnormalities:

☐ Frequency ☐ Dysuria ☐ Urgency ☐ Anuria ☐ Nocturia ☐ Oliguria

☐ Retention ☐ Hematuria ☐ Irrigation ☐ Incontinence ☐ Diapers Used ☐ Penile Discharge

☐ Vaginal Bleeding/Discharge Others (Specify):

Urine Color:

☑ Clear ☐ Transparent ☑ Pale Yellow ☐ Brownish Orange

☐ Pinkish Red ☐ Dark Yellow ☐ Blue or Green ☐ Foamy

☐ Transparent Yellow

Urine Consistency:

☐ Hematuria ☐ Sediment ☐ Odorous ☐ Cloudy

Urinary Catheter:

☐ External Catheter ☐ Foley ☐ Suprapubic ☐ Nephrostomy

Last Catheter Change: mm/dd/yyyy Fr: cc:

MUSCULOSKELETAL

GG0100. Prior Functioning: Everyday Activities: Indicate the patient's usual ability with everyday activities prior to the current illness, exacerbation, or injury.

CODING:

- **3. Independent** – Patient completed the activities by him/herself, with or without an assistive device, with no assistance from a helper
- **2. Needed Some Help** – Patient needed partial assistance from another person to complete activities

- **1. Dependent** – A helper completed the activities for the patient.
- **8. Unknown**
- **9. Not Applicable**

	Independent	Needed Some Help	Dependent	Unknown	Not Applicable
A. Self Care: Code the patient's need for assistance with bathing, dressing, using the toilet, or eating prior to the current illness, exacerbation, or injur	○	○	✓	○	○
B. Indoor Mobility (Ambulation): Code the patient's need for assistance with walking from room to room (with or without a device such as cane, crutch or walker) prior to the current illness, exacerbation, or injury.	○	○	✓	○	○
C. Stairs: Code the patient's need for assistance with internal or external stairs (with or without a device such as cane, crutch, or walker) prior to the current illness, exacerbation or injury.	○	○	✓	○	○
D. Functional Cognition: Code the patient's need for assistance with planning regular tasks, such as shopping or remembering to take medication prior to the current illness, exacerbation, or injury.	○	✓	○	○	○

(GG0110) Prior Device Use

Indicate devices and aids used by the patient prior to the current illness, exacerbation, or injury (**Check all that apply)**

- ☐ **A. Manual wheelchair**
- ☑ **B. Motorized wheelchair and/or scooter**
- ☐ **C. Mechanical lift**
- ☐ **D. Walker**
- ☐ **E. Orthotics/Prosthetics**
- ☐ **Z. None of the above**

GG0170 - MOBILITY

Code the patient's usual performance at SOC/ROC for each activity using the 6-point scale. If activity was not attempted at SOC/ROC, code the reason.
Coding:
Safety and Quality of Performance – If helper assistance is required because patient's performance is unsafe or of poor quality, score according to amount of assistance provided.
Activities may be completed with or without assistive devices.
06. Independent – Patient completes the activity by him/herself with no assistance from a helper.
05. Setup or clean-up assistance – Helper sets up or cleans up; patient completes activity. Helper assists only prior to or following the activity.
04. Supervision or touching assistance – Helper provides verbal cues and/or touching/steadying and/or contact guard assistance as patient completes activity. Assistance may be provided throughout the activity or intermittently.
03. Partial/moderate assistance – Helper does LESS THAN HALF the effort. Helper lifts, holds or supports trunk or limbs, but provides less than half the effort.
02. Substantial/maximal assistance – Helper does MORE THAN HALF the effort. Helper lifts or holds trunk or limbs and provides more than half the effort.
01. Dependent – Helper does ALL of the effort. Patient does none of the effort to complete the activity. Or, the assistance of 2 or more helpers is required for the patient to complete the activity.
If activity was not attempted, code reason:
07. Patient refused
09. Not applicable – Not attempted and the patient did not perform this activity prior to the current illness, exacerbation or injury.
10. Not attempted due to environmental limitations (e.g., lack of equipment, weather constraints)
88. Not attempted due to medical conditions or safety concerns

(GG0170) A. Roll left and right: The ability to roll from lying on back to left and right side, and return to lying on back on the bed.
SOC/ROC Performance

- ○ **(06) Independent**
- ○ **(05) Setup or clean-up assistance**
- ○ **(04) Supervision or touching assistance**
- ● **(03) Partial/moderate assistance**
- ○ **(02) Substantial/maximal assistance**
- ○ **(01) Dependent**
- ○ **(07) Patient refused**
- ○ **(09) Not applicable**
- ○ **(10) Not attempted due to environmental limitations**
- ○ **(88) Not attempted due to medical conditions or safety concerns**
- ○ **(-) Not assessed/no information**

(GG0170) B. Sit to lying: The ability to move from sitting on side of bed to lying flat on the bed.
SOC/ROC Performance

- ○ **(06) Independent**
- ○ **(05) Setup or clean-up assistance**
- ○ **(04) Supervision or touching assistance**
- ○ **(03) Partial/moderate assistance**
- ● **(02) Substantial/maximal assistance**
- ○ **(01) Dependent**
- ○ **(07) Patient refused**
- ○ **(09) Not applicable**
- ○ **(10) Not attempted due to environmental limitations**
- ○ **(88) Not attempted due to medical conditions or safety concerns**
- ○ **(-) Not assessed/no information**

(GG0170) C. Lying to sitting on side of bed: The ability to move from lying on the back to sitting on the side of the bed with feet flat on the floor, and with no back support.
SOC/ROC Performance

- ○ **(06) Independent**
- ○ **(05) Setup or clean-up assistance**
- ○ **(04) Supervision or touching assistance**
- ○ **(03) Partial/moderate assistance**
- ○ **(02) Substantial/maximal assistance**
- ● **(01) Dependent**
- ○ **(07) Patient refused**
- ○ **(09) Not applicable**
- ○ **(10) Not attempted due to environmental limitations**
- ○ **(88) Not attempted due to medical conditions or safety concerns**
- ○ **(-) Not assessed/no information**

(GG0170) D. Sit to stand: The ability to come to a standing position from sitting in a chair, wheelchair, or on the side of the bed.
SOC/ROC Performance

- ○ **(06) Independent**
- ○ **(05) Setup or clean-up assistance**
- ○ **(04) Supervision or touching assistance**
- ○ **(03) Partial/moderate assistance**
- ○ **(02) Substantial/maximal assistance**
- ● **(01) Dependent**
- ○ **(07) Patient refused**
- ○ **(09) Not applicable**
- ○ **(10) Not attempted due to environmental limitations**
- ○ **(88) Not attempted due to medical conditions or safety concerns**
- ○ **(-) Not assessed/no information**

(GG0170) E. Chair/bed-to-chair transfer: The ability to transfer to and from a bed to a chair (or wheelchair).
SOC/ROC Performance

- ○ **(06) Independent**
- ○ **(05) Setup or clean-up assistance**
- ○ **(04) Supervision or touching assistance**
- ○ **(03) Partial/moderate assistance**
- ○ **(02) Substantial/maximal assistance**
- ● **(01) Dependent**
- ○ **(07) Patient refused**
- ○ **(09) Not applicable**
- ○ **(10) Not attempted due to environmental limitations**
- ○ **(88) Not attempted due to medical conditions or safety concerns**
- ○ **(-) Not assessed/no information**

(GG0170) F. Toilet transfer: The ability to get on and off a toilet or commode.
SOC/ROC Performance

- ○ **(06) Independent**
- ○ **(05) Setup or clean-up assistance**
- ○ **(04) Supervision or touching assistance**
- ○ **(03) Partial/moderate assistance**
- ○ **(02) Substantial/maximal assistance**
- ● **(01) Dependent**
- ○ **(07) Patient refused**
- ○ **(09) Not applicable**
- ○ **(10) Not attempted due to environmental limitations**
- ○ **(88) Not attempted due to medical conditions or safety concerns**
- ○ **(-) Not assessed/no information**

(GG0170) G. Car transfer: The ability to transfer in and out of a car or van on the passenger side. Does not include the ability to open/close door or fasten seat belt.
SOC/ROC Performance

- ○ **(06) Independent**
- ○ **(05) Setup or clean-up assistance**
- ○ **(07) Patient refused**
- ○ **(09) Not applicable**

- ○ (04) Supervision or touching assistance
- ○ (03) Partial/moderate assistance
- ○ (02) Substantial/maximal assistance
- ● (01) Dependent
- ○ (10) Not attempted due to environmental limitations
- ○ (88) Not attempted due to medical conditions or safety concerns
- ○ (-) Not assessed/no information

(GG0170) I. Walk 10 feet: Once standing, the ability to walk at least 10 feet in a room, corridor, or similar space. *If SOC/ROC performance is coded 07, 09, 10 or 88, skip to GG0170M, 1 step (curb)*

SOC/ROC Performance

- ○ (06) Independent
- ○ (05) Setup or clean-up assistance
- ○ (04) Supervision or touching assistance
- ○ (03) Partial/moderate assistance
- ○ (02) Substantial/maximal assistance
- ○ (01) Dependent
- ○ (07) Patient refused
- ○ (09) Not applicable
- ○ (10) Not attempted due to environmental limitations
- ● (88) Not attempted due to medical conditions or safety concerns
- ○ (-) Not assessed/no information

(GG0170) J. Walk 50 feet with two turns: Once standing, the ability to walk 50 feet and make two turns.

SOC/ROC Performance

- ○ (06) Independent
- ○ (05) Setup or clean-up assistance
- ○ (04) Supervision or touching assistance
- ○ (03) Partial/moderate assistance
- ○ (02) Substantial/maximal assistance
- ○ (01) Dependent
- ○ (07) Patient refused
- ○ (09) Not applicable
- ○ (10) Not attempted due to environmental limitations
- ○ (88) Not attempted due to medical conditions or safety concerns
- ○ (-) Not assessed/no information

(GG0170) K. Walk 150 feet: Once standing, the ability to walk at least 150 feet in a corridor or similar space.

SOC/ROC Performance

- ○ (06) Independent
- ○ (05) Setup or clean-up assistance
- ○ (04) Supervision or touching assistance
- ○ (03) Partial/moderate assistance
- ○ (02) Substantial/maximal assistance
- ○ (01) Dependent
- ○ (07) Patient refused
- ○ (09) Not applicable
- ○ (10) Not attempted due to environmental limitations
- ○ (88) Not attempted due to medical conditions or safety concerns
- ○ (-) Not assessed/no information

(GG0170) L. Walking 10 feet on uneven surfaces: The ability to walk 10 feet on uneven or sloping surfaces (indoor or outdoor), such as turf or gravel.

SOC/ROC Performance

- ○ (06) Independent
- ○ (05) Setup or clean-up assistance
- ○ (04) Supervision or touching assistance
- ○ (03) Partial/moderate assistance
- ○ (02) Substantial/maximal assistance
- ○ (01) Dependent
- ○ (07) Patient refused
- ○ (09) Not applicable
- ○ (10) Not attempted due to environmental limitations
- ○ (88) Not attempted due to medical conditions or safety concerns
- ○ (-) Not assessed/no information

(GG0170) M. 1 step (curb): The ability to go up and down a curb and/or up and down one step. *If SOC/ROC performance is coded 07, 09, 10 or 88, skip to GG0170P, Picking up object.*

SOC/ROC Performance

- ○ (06) Independent
- ○ (05) Setup or clean-up assistance
- ○ (04) Supervision or touching assistance
- ○ (03) Partial/moderate assistance
- ○ (02) Substantial/maximal assistance
- ○ (01) Dependent
- ○ (07) Patient refused
- ○ (09) Not applicable
- ○ (10) Not attempted due to environmental limitations
- ● (88) Not attempted due to medical conditions or safety concerns
- ○ (-) Not assessed/no information

(GG0170) N. 4 steps: The ability to go up and down four steps with or without a rail. *If SOC/ROC performance is coded 07, 09, 10 or 88, skip to GG0170P, Picking up object.*

SOC/ROC Performance

- ○ (06) Independent
- ○ (05) Setup or clean-up assistance
- ○ (04) Supervision or touching assistance
- ○ (03) Partial/moderate assistance
- ○ (02) Substantial/maximal assistance
- ○ (01) Dependent
- ○ (07) Patient refused
- ○ (09) Not applicable
- ○ (10) Not attempted due to environmental limitations
- ○ (88) Not attempted due to medical conditions or safety concerns
- ○ (-) Not assessed/no information

(GG0170) O. 12 steps: The ability to go up and down 12 steps with or without a rail.

SOC/ROC Performance

- ○ (06) Independent
- ○ (05) Setup or clean-up assistance
- ○ (04) Supervision or touching assistance
- ○ (03) Partial/moderate assistance
- ○ (02) Substantial/maximal assistance
- ○ (01) Dependent
- ○ (07) Patient refused
- ○ (09) Not applicable
- ○ (10) Not attempted due to environmental limitations
- ○ (88) Not attempted due to medical conditions or safety concerns
- ○ (-) Not assessed/no information

(GG0170) P. Picking up object: The ability to bend/stoop from a standing position to pick up a small object, such as a spoon, from the floor.

SOC/ROC Performance

- ○ (06) Independent
- ○ (05) Setup or clean-up assistance
- ○ (04) Supervision or touching assistance
- ○ (03) Partial/moderate assistance
- ○ (02) Substantial/maximal assistance
- ● (01) Dependent
- ○ (07) Patient refused
- ○ (09) Not applicable
- ○ (10) Not attempted due to environmental limitations
- ○ (88) Not attempted due to medical conditions or safety concerns
- ○ (-) Not assessed/no information

(GG0170) Q. Does patient use a wheelchair and/or scooter?

- ○ (0) No _**[GOTO M1600]**
- ● (1) Yes
- ○ Not assessed (no information)

(GG0170) R. Wheel 50 feet with two turns: Once seated in wheelchair/scooter, the ability to wheel at least 50 feet and make two turns.

SOC/ROC Performance

- ○ (06) Independent
- ○ (05) Setup or clean-up assistance
- ○ (04) Supervision or touching assistance
- ○ (03) Partial/moderate assistance
- ● (02) Substantial/maximal assistance
- ○ (01) Dependent
- ○ (07) Patient refused
- ○ (09) Not applicable
- ○ (10) Not attempted due to environmental limitations
- ○ (88) Not attempted due to medical conditions or safety concerns
- ○ (-) Not assessed/no information

(GG0170) RR1. Indicate the type of wheelchair or scooter used.

- ○ (1) Manual
- ● (2) Motorized
- ○ Not assessed (no information)

(GG0170) S. Wheel 150 feet: Once seated in wheelchair/scooter, the ability to wheel at least 150 feet in a corridor or similar space.

SOC/ROC Performance

- ○ (06) Independent
- ○ (05) Setup or clean-up assistance
- ○ (04) Supervision or touching assistance
- ○ (03) Partial/moderate assistance
- ○ (02) Substantial/maximal assistance
- ○ (01) Dependent
- ○ (07) Patient refused
- ○ (09) Not applicable
- ○ (10) Not attempted due to environmental limitations
- ● (88) Not attempted due to medical conditions or safety concerns
- ○ (-) Not assessed/no information

(GG0170) SS1. Indicate the type of wheelchair or scooter used.

- ○ (1) Manual
- ● (2) Motorized
- ○ Not assessed (no information)

Gait and Mobility Abnormalities

- ☐ No Problems Identified
- ☑ Weakness
- ☐ Poor Balance
- ☐ Bed Bound
- ☑ Chair Bound
- ☐ Joint Stiffness
- ☐ Shuffling/Wide-based Gait
- ☐ Orthopedic Aftercare
- ☑ High Risk for Falls
- ☑ Weight Bearing Restriction
- ☑ Poor Conditioning
- ☐ Fracture:
- ☐ Joints Pain:
- ☐ Amputation:
- ☐ Atrophy:
- ☐ Paresthesia:
- ☐ Contractures: Site
- ☑ Foot Drop: ☐ Right ☑ Left

Range of Motion

- ☑ Limited Range of Motion: ☑ RUE ☑ RLE ☑ LUE ☑ LLE
- ☐ Quadriplegia ☐ Hemiplegia ☐ Paraplegia

Comments:

INTEGUMENTARY (SKIN)

M1306. Does this patient have at least one Unhealed Pressure Ulcer/Injury at Stage 2 or Higher or designated as Unstageable?

(Excludes Stage 1 pressure injuries and all healed pressure ulcers/injuries)

- ● 0. No → Skip to M1322, Current Number of Stage 1 Pressure Injuries at SOC/ROC; Skip to M1324, Stage of Most Problematic Unhealed Pressure Ulcer/Injury that is Stageable at DC
- ○ 1. Yes

M1311. Current Number of Unhealed Pressure Ulcers/Injuries at Each Stage

A1. Stage 2: Partial thickness loss of dermis presenting as a shallow open ulcer with a red or pink wound bed, without slough. May also present as an intact or open/ruptured blister. **Number of Stage 2 pressure ulcers**	^
B1. Stage 3: Full thickness tissue loss. Subcutaneous fat may be visible but bone, tendon, or muscle is not exposed. Slough may be present but does not obscure the depth of tissue loss. May include undermining and tunneling. **Number of Stage 3 pressure ulcers**	^
C1. Stage 4: Full thickness tissue loss with exposed bone, tendon, or muscle. Slough or eschar may be present on some parts of the wound bed. Often includes undermining and tunneling. **Number of Stage 4 pressure ulcers**	^
D1. Unstageable: Non-removable dressing/device: Known but not stageable due to non¬removable dressing/device **Number of unstageable pressure ulcers/injuries due to non-removable dressing/device**	^
E1. Unstageable: Slough and/or eschar: Known but not stageable due to coverage of wound bed by slough and/or eschar **Number of unstageable pressure ulcers due to coverage of wound bed by slough and/or eschar**	^
F1. Unstageable: Deep tissue injury Number of unstageable pressure injuries presenting as deep tissue injury	^

(M1322) Current Number of Stage 1 Pressure Injuries:

Intact skin with non-blanchable redness of a localized area usually over a bony prominence. Darkly pigmented skin may not have a visible blanching; in dark skin tones only it may appear with persistent blue or purple hues.

Stage 1 Pressure Injuries **0**

(M1324) Stage of Most Problematic Unhealed Pressure Ulcer/Injury that is Stageable:

(Excludes pressure ulcer/injury that cannot be staged due to a non-removable dressing/device, coverage of wound bed by slough and/or eschar, or deep tissue injury.)

Stage of Most Problematic **Patient has no pressure ulcers or no stageable pressure ulcers**

(M1330) Does this patient have a Stasis Ulcer?

- ● No **[GO TO M1340]**
- ○ Yes, patient has BOTH observable and unobservable stasis ulcers
- ○ Yes, patient has observable stasis ulcers ONLY
- ○ Yes, patient has unobservable stasis ulcers ONLY (known but not observable due to non¬removable dressing/device) **[GO TO M1340]**

(M1332) Current Number of Stasis Ulcer(s) that are Observable:

- ○ One
- ○ Two
- ○ Three
- ○ Four or more

(M1334) Status of Most Problematic Stasis Ulcer that is Observable

- ○ Fully granulating
- ○ Early/partial granulation
- ○ Not healing

(M1340) Does this patient have a Surgical Wound?

- ● No **[GO TO N0415]**
- ○ Yes, patient has at least one observable surgical wound
- ○ Surgical wound known but not observable due to non-removable dressing/device **[GO TO N0415]**

(M1342) Status of Most Problematic Surgical Wound that is Observable

- ○ Newly epithelialized
- ○ Fully granulating
- ○ Early/partial granulation
- ○ Not healing

Skin Color

- ☑ Within Normal Limits
- ☐ Pale
- ☐ Jaundice
- ☐ Cyanotic

Skin Turgor:

- ☐ Good
- ☑ Fair
- ☐ Poor

Skin Abnormalities:

- ☑ Dry
- ☑ Warm
- ☐ Cool
- ☐ Rash
- ☐ Ulcer
- ☐ Ostomy
- ☐ Incision
- ☐ Diaphoretic
- ☐ Others (Specify):

Norton Scale for Pressure Ulcer Risk Assessment

	Good (4)	Fair (3)	Poor (2)	Very Bad (1)

Physical Condition	◯ Good	◯ Fair	● Poor	◯ Very Bad
Mental Condition	● Alert	◯ Apathetic	◯ Confused	◯ Stuporous
Activity	◯ Ambulant	◯ Walks With Help	● Chairbound	◯ Bedfast
Mobility	◯ Full	◯ Slightly Impaired	● Very Limited	◯ Immobile
Incontinence	● None	◯ Occassional	◯ Usually Urinary	◯ Urinary and Fecal
			Total Score:	14

Wound

Fetch Wounds From Previous Wound Note

PAIN ASSESSMENT

J0510. Pain Effect on Sleep

Ask patient: "Over the past 5 days, how much of the time has pain made it hard for you to sleep at night"

- ◯ **0. Does not apply – I have not had any pain or hurting in the past 5 days**
- ◯ **1. Rarely or not at all**
- ◯ **2. Occasionally**
- ● **3. Frequently**
- ◯ **4. Almost constantly**
- ◯ **8. Unable to answer**

J0520. Pain Interference with Therapy Activities

Ask patient: "Over the past 5 days, how often have you limited your participation in rehabilitation therapy sessions due to pain?"

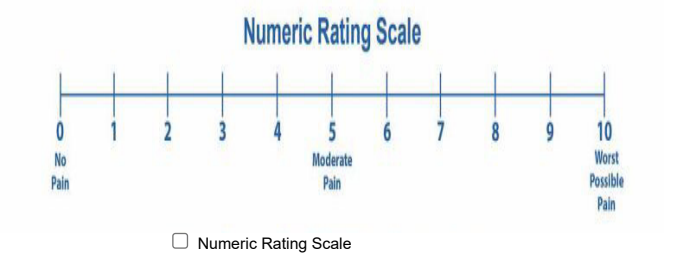
- ● **0. Does not apply – I have not received rehabilitation therapy in the past 5 days**
- ◯ **1. Rarely or not at all**
- ◯ **2. Occasionally**
- ◯ **3. Frequently**
- ◯ **4. Almost constantly**
- ◯ **8. Unable to answer**

J0530. Pain Interference with Day-to-Day Activities

Ask patient: "Over the past 5 days, how often you have limited your day-to-day activities (excluding rehabilitation therapy session) because of pain?"

- ◯ **1 - Rarely or not at all**
- ◯ **2 - Occasionally**
- ◯ **3 - Frequently**
- ● **4 - Almost Constantly**
- ◯ **Unable to answer**

Pain



8

Collected Using: ☐ Face Scale ☐ Numeric Rating Scale

Pain Location and Description:

1

Location	Start Date	Activity	What makes pain worse?
BLE.	02/18/2026	With Activity×	Activity×

Intensity	Duration	What makes pain better?	Does pain affect quality of life?
8×		Med/Rest×	Yes.
	Unable to Assess ☑		Unable to Assess ☐

Description	Frequency	Type of Intervention
Throbbing×	Constant×	Opiod Analgesic×

How well it is working	Status
Fairly Well×	Treat×

EMOTIONAL/MOOD

(D0150) Patient Mood Interview (PHQ-2 to 9)

Say to patient: **"Over the last 2 weeks, have you been bothered by any of the following problems?"**
If symptom is present, **enter 1 (yes) in column 1 , Symptom Presence** . If yes in column 1, then ask the patient: **"About how often have you been bothered by this?"**
Read and show the patient a card with the symptom frequency choices. Indicate response in column 2, Symptom Frequency

1) Symptom Presence
- **0. No (enter 0 in column 2)**
- **1. Yes (enter 0-3 in column 2)**
- **9. No response (leave column 2 blank)**

2) Symptom Frequency
- 0 - **Never or 1 day**
- 1 - **2-6 days (several days)**
- 2 - **7-11 days (half or more of the days)**
- 3 - **12-14 days (nearly every day)**

	Presence	Frequency
A. Little interest or pleasure in doing things	Yes ×	7-11 days (half or more of the days) ×
B. Feeling down, depressed, or hopeless	Yes ×	7-11 days (half or more of the days) ×
If either **D0150A2 or D0150B2** is coded **2 or 3** , **CONTINUE** asking the questions below. If not, **END the PHQ interview**		
C. Trouble falling or staying asleep, or sleeping too much	No ×	Never or 1 day
D. Feeling tired or having little energy	Yes ×	12-14 days (nearly every day) ×
E. Poor appetite or overeating	No ×	Never or 1 day
F. Feeling bad about yourself – or that you are a failure or have let yourself or your family down	No ×	Never or 1 day
G. Trouble concentrating on things, such as reading the newspaper or watching television	No ×	Never or 1 day
H. Moving or speaking so slowly that other people could have noticed. Or the opposite – being so fidgety or restless that you have been moving around a lot more than usual	Yes ×	12-14 days (nearly every day) ×
I. Thoughts that you would be better off dead, or of hurting yourself in some way	No ×	Never or 1 day
D0160. Total Severity Score Add scores for all frequency responses in Column 2, Symptom Frequency. Total score must be between 02 and 27. Enter 99 if unable to complete interview (i.e., Symptom Frequency is blank for 3 or more required items)		10

D0700. Social Isolation

How often do you feel lonely or isolated from those around you?

- [x] 0 - Never
- [ ] 1 - Rarely
- [ ] 2 - Sometimes
- [ ] 3 - Often
- [ ] 4 - Always
- [ ] 7 - Patient declines to respond
- [ ] 8 - Patient unable to respond

(M1740) Cognitive, behavioral, and psychiatric symptoms that are demonstrated at least once a week (Reported or Observed): (Mark all that apply.

- [ ] 1 - Memory deficit: failure to recognize familiar persons/places, inability to recall events of past 24 hours, significant memory loss so that supervision is required
- [x] 2 - Impaired decision-making: failure to perform usual ADLs or IADLs, inability to appropriately stop activities, jeopardizes safety through actions
- [ ] 3 - Verbal disruption: yelling, threatening, excessive profanity, sexual references, etc.
- [ ] 4 - Physical aggression: aggressive or combative to self and others (for example, hits self, throws objects, punches, dangerous maneuvers with wheelchair or other objects)
- [ ] 5 - Disruptive, infantile, or socially inappropriate behavior (excludes verbal actions)
- [ ] 6 - Delusional, hallucinatory, or paranoid behavior
- [ ] 7 - None of the above behaviors demonstrated

(M1745) Frequency of Disruptive Behavior Symptoms (Reported or Observed):

- [ ] 0 - Never
- [ ] 1 - Less than once a month
- [ ] 2 - Once a month
- [ ] 3 - Several times each month
- [x] 4 - Several times a week
- [ ] 5 - At least daily

FUNCTIONAL STATUS

(M1800) Grooming

Current ability to tend safely to personal hygiene needs (specifically: washing face and hands, hair care, shaving or make up, teeth or denture care, or fingernail care)

- [ ] 0 - Able to groom self unaided, with or without the use of assistive devices or adapted methods.
- [ ] 1 - Grooming utensils must be placed within reach before able to complete grooming activities.
- [ ] 2 - Someone must assist the patient to groom self.
- [x] 3 - Patient depends entirely upon someone else for grooming needs

(M1810)

Current **Ability to Dress Upper Body** safely (with or without dressing aids) including undergarments, pullovers, front-opening shirts and blouses, managing zippers, buttons, and snaps:

- [ ] 0 - Able to get clothes out of closets and drawers, put them on and remove them from the upper body without assistance.
- [ ] 1 - Able to dress upper body without assistance if clothing is laid out or handed to the patient.
- [x] 2 - Someone must help the patient put on upper body clothing.
- [ ] 3 - Patient depends entirely upon another person to dress the upper body.

(M1820)

Current **Ability to Dress Lower Body** safely (with or without dressing aids) including undergarments, slacks, socks or nylons, shoes:

- [ ] 0 - Able to obtain, put on, and remove clothing and shoes without assistance.
- [ ] 1 - Able to dress lower body without assistance if clothing and shoes are laid out or handed to the patient.
- [ ] 2 - Someone must help the patient put on undergarments, slacks, socks or nylons, and shoes.
- [x] 3 - Patient depends entirely upon another person to dress lower body.

(M1830) Bathing:

Current ability to wash entire body safely. **Excludes grooming (washing face, washing hands, and shampooing hair).**

- [ ] 0 - Able to bathe self in shower or tub independently, including getting in and out of tub/shower.
- [ ] 1 - With the use of devices, is able to bathe self in shower or tub independently, including getting in and out of the tub/shower.
- [ ] 2 - Able to bathe in shower or tub with the intermittent assistance of another person: (a) for intermittent supervision or encouragement or reminders, OR (b) to get in and out of the shower or tub, OR (c) for washing difficult to reach areas.
- [x] 3 - Able to participate in bathing self in shower or tub, but requires presence of another person throughout the bath for assistance or supervision.
- [ ] 4 - Unable to use the shower or tub, but able to bathe self independently with or without the use of devices at the sink, in chair, or on commode.
- [ ] 5 - Unable to use the shower or tub, but able to participate in bathing self in bed, at the sink, in bedside chair, or on commode, with the assistance or supervision of another person
- [ ] 6 - Unable to participate effectively in bathing and is bathed totally by another person.

(M1840) Toilet Transferring:

Current ability to get to and from the toilet or bedside commode safely and transfer on and off toilet/commode

- ○ **0 - Able to get to and from the toilet and transfer independently with or without a device**
- ○ **1 - When reminded, assisted, or supervised by another person, able to get to and from the toilet and transfer.**
- ✓ **2 - Unable to get to and from the toilet but is able to use a bedside commode (with or without assistance).**
- ○ **3 - Unable to get to and from the toilet or bedside commode but is able to use a bedpan/urinal independently**
- ○ **4 - Is totally dependent in toileting.**

(M1845) Toileting Hygiene:

Current ability to maintain perineal hygiene safely, adjust clothes and/or incontinence pads before and after using toilet, commode, bedpan, urinal. If managing ostomy, includes cleaning area around stoma, but not managing equipment.

- ○ **0 - Able to manage toileting hygiene and clothing management without assistance.**
- ○ **1 - Able to manage toileting hygiene and clothing management without assistance if supplies/implements are laid out for the patient.**
- ✓ **2 - Someone must help the patient to maintain toileting hygiene and/or adjust clothing.**
- ○ **3 - Patient depends entirely upon another person to maintain toileting hygiene**

(M1850) Transferring:

Current ability to move safely from bed to chair, or ability to turn and position self in bed if patient is bedfast

- ○ **0 - Able to independently transfer.**
- ○ **1 - Able to transfer with minimal human assistance or with use of an assistive device.**
- ○ **2 - Able to bear weight and pivot during the transfer process but unable to transfer self.**
- ✓ **3 - Unable to transfer self and is unable to bear weight or pivot when transferred by another person**
- ○ **4 - Bedfast, unable to transfer but is able to turn and position self in bed.**
- ○ **5 - Bedfast, unable to transfer and is unable to turn and position self**

(M1860) Ambulation/Locomotion

Current ability to walk safely, once in a standing position, or use a wheelchair, once in a seated position, on a variety of surfaces.

- ○ **0 - Able to independently walk on even and uneven surfaces and negotiate stairs with or without railings (specifically: needs no human assistance or assistive device).**
- ○ **1 - With the use of a one-handed device (for example, cane, single crutch, hemi-walker), able to independently walk on even and uneven surfaces and negotiate stairs with or without railings.**
- ○ **2 - Requires use of a two-handed device (for example, walker or crutches) to walk alone on a level surface and/or requires human supervision or assistance to negotiate stairs or steps or uneven surfaces.**
- ○ **3 - Able to walk only with the supervision or assistance of another person at all times.**
- ✓ **4 - Chairfast, unable to ambulate but is able to wheel self independently.**
- ○ **5 - Chairfast, unable to ambulate and is unable to wheel self**
- ○ **6 - Bedfast, unable to ambulate or be up in a chair.**

GG0130 - SELF CARE

Code the patient's usual performance at SOC/ROC for each activity using the 6-point scale. If activity was not attempted at SOC/ROC, code the reason.
Coding:
Safety and Quality of Performance – If helper assistance is required because patient's performance is unsafe or of poor quality, score according to amount of assistance provided.
Activities may be completed with or without assistive devices.

06. Independent – Patient completes the activity by him/herself with no assistance from a helper.
05. Setup or clean-up assistance – Helper sets up or cleans up; patient completes activity. Helper assists only prior to or following the activity.
04. Supervision or touching assistance – Helper provides verbal cues and/or touching/steadying and/or contact guard assistance as patient completes activity. Assistance may be provided throughout the activity or intermittently.
03. Partial/moderate assistance – Helper does LESS THAN HALF the effort. Helper lifts, holds or supports trunk or limbs, but provides less than half the effort.
02. Substantial/maximal assistance – Helper does MORE THAN HALF the effort. Helper lifts or holds trunk or limbs and provides more than half the effort.
01. Dependent – Helper does ALL of the effort. Patient does none of the effort to complete the activity. Or, the assistance of 2 or more helpers is required for the patient to complete the activity.

If activity was not attempted, code reason:

07. Patient refused
09. Not applicable – Not attempted and the patient did not perform this activity prior to the current illness, exacerbation or injury.
10. Not attempted due to environmental limitations (e.g., lack of equipment, weather constraints)
88. Not attempted due to medical conditions or safety concerns

(GG0130)SOC/ROC Performance

A. Eating: The ability to use suitable utensils to bring food and/or liquid to the mouth and swallow food and/or liquid once the meal is placed before the patient.
B. Oral Hygiene: The ability to use suitable items to clean teeth. Dentures (if applicable): The ability to insert and remove dentures into and from mouth, and manage denture soaking and rinsing with use of equipment.
C. Toileting Hygiene: The ability to maintain perineal hygiene, adjust clothes before and after voiding or having a bowel movement. If managing an ostomy, include wiping the opening but not managing equipment.
E. Shower/bathe self: The ability to bathe self, including washing, rinsing, and drying self (excludes washing of back and hair). Does not include transferring in/out of tub/shower.
F. Upper body dressing: The ability to dress and undress above the waist; including fasteners, if applicable.
G. Lower body dressing: The ability to dress and undress below the waist, including fasteners; does not include footwear.
H. Putting on/taking off footwear: The ability to put on and take off socks and shoes or other footwear that is appropriate for safe mobility; including fasteners, if applicable.

	A. Eating	B. Oral Hygiene	C. Toileting Hygiene	E. Shower /bathe self	F. Upper body dressing	G. Lower body dressing	H. Putting on /taking off footwear
(06) Independent	○	○	○	○	○	○	○
(05) Setup or clean-up assistance	○	○	○	○	○	○	○
(04) Supervision or touching assistance	○	○	○	○	○	○	○
(03) Partial/moderate assistance	○	✓	✓	○	✓	○	○
(02) Substantial/maximal assistance	○	○	○	✓	○	✓	✓
(01) Dependent	✓	○	○	○	○	○	○
(07) Patient refused	○	○	○	○	○	○	○
(09) Not applicable	○	○	○	○	○	○	○
(10) Not attempted due to environmental limitations	○	○	○	○	○	○	○
(88) Not attempted due to medical conditions or safety concerns	○	○	○	○	○	○	○
(-) Not assessed/no information	○	○	○	○	○	○	○

DME and Supplies

DME Lookup

Durable Medical Equipment:
Bath bench, Walker, Wheelchair

#18B ACTIVITIES PERMITTED

- ( ) 1- Complete bedrest
- ( ) 2 - Bedrest/BRP
- (✓) 3 - Up as tolerated
- (✓) 4 - Transfer bed/chair
- ( ) 5 - Exercises prescribed
- (✓) 6 - Partial weight bearing
- ( ) 7 - Independent in home
- ( ) 8 - Crutches
- ( ) 9 - Cane
- (✓) A - Wheelchair
- ( ) B - Walker
- ( ) C - No restrictions
- ( ) D - Other (specify)

DIAGNOSIS

M1100. Patient Living Situation. Which of the following best describes the patient's residential circumstance and availability of assistance?

Living Arrangement	Around the clock	Regular daytime	Regular Night time	Short-term Assistance	No Assistance available
A. Patient lives alone	( )	( )	( )	( )	( )
B. Patient lives with other person(s) in the home	(✓)	( )	( )	( )	( )
C. Patient lives in congregate situation (for example, assisted living, residential care home)	( )	( )	( )	( )	( )

Primary Caregiver
Jayson Hylton

Relationship to Patient
Spouse

Contact Number
951 380 0797

Comments
Very skilled and passionate.

Name Of Facility

Phone:

Able to safely care for patient?
(✓) Yes ( ) No

Home Environment / Home Safety Assessment

- [ ] Architectural Barriers
- [ ] Medications are improperly stored
- [ ] No telephone
- [x] Cluttered surroundings
- [ ] Inadequate railing
- [ ] Unsafe oxygen use
- [ ] No fire extinguisher
- [ ] Inadequate electricity / cooling / heating
- [ ] Inadequate supply of water
- [ ] Inadequate sanitation / plumbing
- [ ] Inadequate smoke detectors on all levels of home
- [ ] Unsafe storage of equipments
- [ ] No emergency exit plan
- [ ] Rodent / insect infestation
- [ ] Unsafe electrical wiring
- [ ] Nonworking stove / refrigerator
- [ ] Exposed flammables
- [ ] Inadequate Lighting

Other:

Type of Assistance Patient Receives - other than from Home Health Agency Staff:

	Family/Friends	Provider Services	Paid Caregiver	Volunteer Organizations
ADL(Bathing, dressing, toileting, eating/feeding, bowel)	[x]	[ ]	[ ]	[ ]
IADL (Meds, meals, house-keeping, laundry, telephone, shipping, finances)	[x]	[ ]	[ ]	[ ]
Psychosocial Support	[x]	[x]	[ ]	[ ]
Assistance with doctors appointment, delivery of medications	[x]	[ ]	[ ]	[ ]
Management of Finances	[x]	[ ]	[ ]	[ ]

Comments:

Safety Hazards Affecting Patient:

- [ ] Stairs
- [ ] Inadequate lighting, heating and cooling
- [ ] Insect/Rodent Infestation
- [ ] No gas/electric appliance
- [ ] No hazards identified
- [ ] No running water, plumbing
- [x] Narrow or obstructed walkway
- [ ] Lack of fire safety devices
- [x] Cluttered/soiled living area

Others:

(M2102) Types and Sources of Assistance:

Determine the ability and willingness of non-agency caregivers (such as family members, friends, or privately paid caregivers) to provide assistance for the following activities, if assistance is needed. Excludes all care by your agency staff

F. Supervision and Safety (for example, due to cognitive impairment)

- ( ) No assistance needed –patient is independent or does not have needs in this area
- (✓) Non-agency caregiver(s) currently provide assistance
- ( ) Non-agency caregiver(s) need training/ supportive services to provide assistance
- ( ) Non-agency caregiver(s) are not likely to provide assistance OR it is unclear if they will provide assistance
- ( ) Assistance needed, but no non-agency caregiver(s) available

Safety Measures

- ( ) 1 - Bleeding precautions
- ( ) 2 - O_2 precautions
- ( ) 3 - Seizure precautions
- (✓) 4 - Fall precautions
- ( ) 5 - Aspiration precautions
- ( ) 6 - Siderails up
- ( ) 7 - Elevate head of bed
- ( ) 8 - 24 hr. supervision
- (✓) 9- Clear Pathways up
- (✓) 10 - Lock w/c with transfers
- (✓) 11 - Infection control measures
- ( ) 12 - Walker/cane
- ( ) 13 - Sharps Precaution
- (✓) 14 - Other:

911/ER protocol, universal precaution

(M1021/1023) Diagnoses and Symptom Control

List each diagnosis for which the patient is receiving home care in Column 1, and enter its ICD-10-CM code at the level of highest specificity in Column 2 (diagnosis codes only – no surgical or procedure codes allowed). Diagnoses are listed in the order that best reflects the seriousness of each condition and supports the disciplines and services provided. Rate the degree of symptom control for each condition in Column 2. ICD-10-CM sequencing requirements must be followed if multiple coding is indicated for any diagnoses.

Code each row according to the following directions for each column:

- **Column** 1: Enter the description of the diagnosis. Sequencing of diagnoses should reflect the seriousness of each condition and support the disciplines and services provided. Column 2: Enter the ICD-10-CM code for the condition described in Column 1 – no surgical or procedure codes allowed. Codes must be entered at the level of highest specificity and ICD-10-CM coding rules and sequencing requirements must be followed. Note that external cause codes (ICD-10-CM codes beginning with V, W, X, or Y) may not be reported in M1021 (Primary Diagnosis) but may be reported in M1023 (Secondary Diagnoses). Also note that when a Z-code is reported in

- **Column 2 :** the code for the underlying condition can often be entered in Column 2, as long as it is an active on-going condition impacting home health care. Rate the degree of symptom control for the condition listed in Column 1. Do not assign a symptom control rating if the diagnosis code is a V, W, X, Y or Z-code. Choose one value that represents the degree of symptom control appropriate for each diagnosis using the following scale: 0 - Asymptomatic, no treatment needed at this time 1 - Symptoms well controlled with current therapy 2 - Symptoms controlled with difficulty, affecting daily functioning; patient needs ongoing monitoring 3 - Symptoms poorly controlled; patient needs frequent adjustment in treatment and dose monitoring 4 - Symptoms poorly controlled; history of re-hospitalizations Note that the rating for symptom control in Column 2 should not be used to determine the sequencing of the diagnoses listed in Column 1. These are separate items and sequencing may not coincide.

OASIS IS ALREADY EXPORTED, PRIMARY DIAGNOSIS AND OTHER DIAGNOSIS FROM 1 TO 5 ARE NOW DISABLED

	Onset Date	Severity						
PRIMARY DIAGNOSIS G62.89 - Other specified polyneuropathies	02/18/2026	○ 0	○ 1	○ 2	● 3	○ 4	○ ^	○ Onset ● Exacerbate
OTHER DIAGNOSIS - 1 G56.03 - Carpal tunnel syndrome, bilateral upper limbs	02/18/2026	○ 0	○ 1	● 2	○ 3	○ 4	○ ^	● Onset ○ Exacerbate
OTHER DIAGNOSIS - 2 G89.29 - Other chronic pain	02/18/2026	○ 0	○ 1	● 2	○ 3	○ 4	○ ^	● Onset ○ Exacerbate
OTHER DIAGNOSIS - 3 M62.81 - Muscle weakness (generalized)	02/18/2026	○ 0	○ 1	● 2	○ 3	○ 4	○ ^	● Onset ○ Exacerbate
OTHER DIAGNOSIS - 4 M21.372 - Foot drop, left foot	02/18/2026	○ 0	○ 1	● 2	○ 3	○ 4	○ ^	● Onset ○ Exacerbate
OTHER DIAGNOSIS - 5 I10. - Essential (primary) hypertension	02/18/2026	○ 0	○ 1	● 2	○ 3	○ 4	○ ^	● Onset ○ Exacerbate
E87.5 - Hyperkalemia	02/18/2026	○ 0	○ 1	● 2	○ 3	○ 4	○ ^	● Onset ○ Exacerbate
D75.839 - Thrombocytosis, unspecified	02/18/2026	○ 0	○ 1	● 2	○ 3	○ 4	○ ^	● Onset ○ Exacerbate
R56.9 - Unspecified convulsions	02/18/2026	○ 0	○ 1	● 2	○ 3	○ 4	○ ^	● Onset ○ Exacerbate
F41.9 - Anxiety disorder, unspecified	02/18/2026	○ 0	○ 1	● 2	○ 3	○ 4	○ ^	● Onset ○ Exacerbate

	Onset Date	Severity	
F32.A - Depression, unspecified	02/18/2026	0 1 **2** 3 4 ^	**Onset** Exacerbate
R41.3 - Other amnesia	02/18/2026	0 1 **2** 3 4 ^	**Onset** Exacerbate
R29.6 - Repeated falls	02/18/2026	0 1 **2** 3 4 ^	**Onset** Exacerbate
Z91.81 - History of falling	02/18/2026	0 1 2 3 4 **^**	**Onset** Exacerbate
Z99.89 - Dependence on other enabling machines and devices	02/18/2026	0 1 2 3 4 **^**	**Onset** Exacerbate

(M1028) Active Diagnoses – Comorbidities and Co-existing Conditions – Check all that apply See OASIS Guidance Manual for a complete list of relevant ICD-10 codes.

- [ ] Diabetes Mellitus (DM)
- [ ] Peripheral Vascular Disease (PVD) or Peripheral Arterial Disease (PAD)
- [x] None of the above

(M1033) Risk for Hospitalization: Which of the following signs or symptoms characterize this patient as at risk for hospitalization? (Mark all that apply.)

- [x] 1 - History of falls (2 or more falls – or any fall with an injury – in the past 12 months)
- [ ] 2 - Unintentional weight loss of a total of 10 pounds or more in the past 12 months
- [ ] 3 - Multiple hospitalizations (2 or more) in the past 6 months
- [ ] 4 - Multiple emergency department visits (2 or more) in the past 6 months
- [ ] 5 - Decline in mental, emotional, or behavioral status in the past 3 months
- [ ] 6 - Reported or observed history of difficulty complying with any medical instructions (for example, medications, diet, exercise) in the past 3 months
- [x] 7 - Currently taking 5 or more medications
- [ ] 8 - Currently reports exhaustion
- [ ] 9 - Other risk(s) not listed in 1 - 8
- [ ] 10 - None of the above

Additional Risk Information:

Prior Hospitalization

	Number of times
Yes No	1

Hospitalization Risk Assessment

Prior Pattern: Check all that apply

- [x] > 1 Hospitalizations or ER visits in the past 12 months
- [x] History of falls

Chronic conditions: Check all that apply (M1020/1022/1024)

- [ ] HF
- [ ] Chronic skin ulcers (Wound consult if indicated for any wounds)
- [ ] Diabetes
- [ ] HIV/AIDS
- [ ] COPD

Risk factors: (Check all that apply)

- [ ] Discharged from hospital or skilled nursing facility
- [x] Help with managing medications needed ► ★
- [x] More than 2 secondary diagnoses
- [ ] Non-compliance with medication regimen ♦ ★
- [ ] Low socioeconomic status or financial concerns ♦
- [ ] Confusion ♦ ★
- [ ] Lives alone ►♦
- [ ] Pressure ulcer ★
- [ ] Inadequate support network ♦
- [ ] Stasis ulcer ★
- [x] ADL assistance needed ►
- [ ] Overall Poor Status/Prognosis ■
- [x] Home safety risks ► ♦
- [ ] Low literacy level ♦
- [ ] Dyspnea ► ★
- [x] Depression ♦
- [x] ►Consider Therapy referral (PT, OT, ST)
- [x] ♦Consider MSW referral
- [ ] ■ Consider Hospice referral
- [ ] ★Consider RN referral, if not ordered

Total number of checked boxes is

9 Your agency may want to select a threshold score to target patients at high risk.

(For example: 5 or greater risk factors may indicate that the patient is at risk for hospitalization. Note: This number is for convenience only and has not been tested or validated. The agency may modify the score based upon the needs of their patient population.)

Carry out patient specific interventions as appropriate/ordered, if patient is at risk for hospitalization: (Coordinate with M2250)

Referrals:

- [x] SN
- [x] PT
- [ ] OT
- [ ] ST
- [ ] MSW
- [ ] HHA
- [ ] Dietary Consultant

Other

Consider implementing any of the following interventions, if patient is at risk for hospitalization:

- [x] Medication Management
- [x] Medication Reconciliation
- [x] Assess patient's: knowledge, ability, resources and adherence
- [x] Education
- [x] Patient/family education
- [ ] Enrollment into a disease management program

(Specify)

- [ ] Hospice/Palliative Referral
- [ ] Phone Monitoring
- [ ] Front-loading Visits
- [ ] Individualized Patient Emergency Care Plan
- [ ] Telemonitoring
- [ ] Care Coordination (physicians, hospitals, nursing homes ...)
- [x] Fall Prevention Program
- [ ] Wound Care

Immunizations (M1040, M1045, M1050, M1055)

- [ ] Influenza
- [ ] Pneumococcal

Fall Prevention Program
Other

MAHC 10* - FALL RISK ASSESSMENT TOOL

	Yes	No
Age 65+	○	✓
Diagnosis (3 or more co-existing) Includes only documented medical diagnosis	✓	○
Prior history of falls within 3 months An unintentional change in position resulting in coming to rest on the ground or at a lower level	○	✓
Incontinence Inability to make it to the bathroom or commode in timely manner. Includes frequency, urgency, and/or nocturia.	○	✓
Visual impairment Includes but not limited to, macular degeneration, diabetic retinopathies, visual field loss, age related changes, decline in visual acuity, accommodation, glare tolerance, depth perception, and night vision or not wearing prescribed glasses or having the correct prescription.	○	✓
Impaired functional mobility May include patients who need help with IADLS or ADLS or have gait or transfer problems, arthritis, pain, fear of falling, foot problems, impaired sensation, impaired coordination or improper use of assistive devices.	✓	○
Environmental hazards May include but not limited to, poor illumination, equipment tubing, inappropriate footwear, pets, hard to reach items, floor surfaces that are uneven or cluttered, or outdoor entry and exits.	✓	○
Poly Pharmacy (4 or more prescriptions - any type) All PRESCRIPTIONS including prescriptions for OTC meds. Drugs highly associated with fall risk include but not limited to, sedatives, antidepressants, tranquilizers, narcotics, antihypertensives, cardiac meds, corticosteroids, anti-anxiety drugs, anticholinergic drugs, and hypoglycemic drugs.	✓	○
Pain affecting level of function Pain often affects an individual's desire or ability to move or pain can be a factor in depression or compliance with safety recommendations.	✓	○
Cognitive impairment Could include patients with dementia, Alzheimer's or stroke patients or patients who are confused, use poor judgment, have decreased comprehension, impulsivity, memory deficits. Consider patients ability to adhere to the plan of care.	✓	○
A score of 4 or more is considered at risk for falling	6	

MEDICATION AND ALLERGIES

(N0415) High-Risk Drug Classes: Use and Indication:

	1. Is Taking	2. Indication Noted
	1. Is taking Check if the patient is taking any medications by pharmacological classification, not how it is used, in the following classes	2. Indication noted If Column 1 is checked, check if there is an indication noted for all medications in the drug class
A. Antipsychotic	○	■
E. Anticoagulant	○	■
F. Antibiotic	○	■
H. Opioid	✓	✓
I. Antiplatelet	○	■
J. Hypoglycemic (including insulin)	○	■
Z. None of the Above	○	

Medication

☐	Start Date	Medication	Strength / Dosage / Frequency	Route	Classification/Indication	Status
☐	02/18/2026	Losartan (Oral Pill)	100 mg 1 Tab Once a day	By mouth	ANGIOTENSIN II INHIBITOR /	New
☐	02/18/2026	Magnesium oxide	400 mg 1 tab once at bedtime	By mouth	Mineral supplement /	Old
☐	02/18/2026	KEPPRA	750 mg 1 Tab Every six hours	By mouth	ANTICONVULSANTS /	New
☐	02/18/2026	NORCO	325-5 mg 1 Tab Four times a day	By mouth	OPIOID ANALGESICS / for pain	Old
☐	02/18/2026	diazepam	2 mg Take 1 tablet Once a day	By mouth	Benzodiazepine /	Old

	Start Date		Strength / Dosage / Frequency	Route	Classification/Indication	Status
☐	02/18/2026	gabapentin	800 mg Take 1 tablet Four times a day	By mouth	Salicylate-containing product /	Old
☐	02/18/2026	buspirone	15 mg Take 1 tablet Twice a day	By mouth	Anxiolytic /	Old

Medication Remarks
Medication remarks...

☐ Discontinued Medications

Start Date	Medication	Strength / Dose / Frequency / Route	Classification/Indication	Laboratory	Status	Disc Date	Remarks

Does patient have IV access? ○ Yes ● No

(M2001) Drug Regimen Review

Did a complete drug regimen review identify potential clinically significant medication issues?

- ● **No – No issues found during review _[GO TO M2010]**
- ○ **Yes – Issues found during review**
- ○ **Not assessed/no information**
- ○ **NA – Patient is not taking any medications _[GO TO M2102]**

(M2003) Medication Follow-up

Did the agency contact a physician (or physician-designee) by midnight of the next calendar day and complete prescribed/recommended actions in response to the identified potential clinically significant medication issues?

- ○ **No**
- ○ **Yes**
- ○ **Not assessed/no information**

M2010. Patient/Caregiver High-Risk Drug Education

Has the patient/caregiver received instruction on special precautions for all high-risk medications (such as hypoglycemics, anticoagulants, etc.) and how and when to report problems that may occur?

- ○ **NO**
- ● **YES**
- ○ **NA - Patient not taking any high-risk drugs OR patient/caregiver fully knowledgeable about special precautions associated with all high-risk medications**

M2020. Management of Oral Medications

Patient's current ability to prepare and take all oral medications reliably and safely, including administration of the correct dosage at the appropriate times/intervals. Excludes injectable and IV medications. (NOTE: This refers to ability, not compliance or willingness.)

- ○ **Able to independently take the correct oral medication(s) and proper dosage(s) at the correct times.**
- ○ **Able to take medication(s) at the correct times if: (a) individual dosages are prepared in advance by another person; OR (b) another person develops a drug diary or chart.**
- ● **Able to take medication(s) at the correct times if given reminders by another person at the appropriate times.**
- ○ **Unable to take medication unless administered by another person**
- ○ **NA - No oral medications prescribed**

M2030. Management of Injectable Medications

Patient's current ability to prepare and take all prescribed injectable medications reliably and safely, including administration of correct dosage at the appropriate times/intervals. Excludes IV medications.

- ○ **Able to independently take the correct medication(s) and proper dosage(s) at the correct times**
- ○ **Able to take injectable medication(s) at the correct times if: a. individual syringes are prepared in advance by another person; OR b. another person develops a drug diary or chart.**
- ○ **Able to take medication(s) at the correct times if given reminders by another person based on the frequency of the injection**
- ○ **Unable to take injectable medication unless administered by another person**
- ● **NA - No injectable medications prescribed.**

Allergies

☐ Add Allergies

Name	Reaction	Start Date	
Penicillin	Rash	04/11/2025	☐

(O0110). Special Treatments, Procedures, and Programs

Check all of the following treatments, procedures, and programs that apply at discharge.

	On Admission (Check All That Apply)
Cancer Treatment	
A1. Chemotherapy	☐
A2. IV	☐
A3. Oral	☐
A10.Other	☐
B1. Radiation	☐
Respiratory Therapies	
C1. Oxygen Therapy	☐
C2. Continuous	☐
C3. Intermittent	☐
C4. High-Concentration	☐
D1. Suctioning	☐
D2. Scheduled	☐
D3. As needed	☐
E1. Tracheostomy Care	☐
F1. Invasive Mechanical Ventilator	☐

G1. Non-invasive Mech. Vent.	☐
G2. BiPAP	☐
G3. CPAP	☐
Others	
H1. IV Medications	☐
H2. Vasoactive medications	☐
H3. Antibiotics	☐
H4. Anticoagulation	☐
H10.Other	☐
I1. Transfusions	☐
J1. Dialysis	☐
J2. Hemodialysis	☐
J3. Peritoneal dialysis	☐
O1. IV Access	☐
O2. Peripheral	☐
O3. Mid-line	☐
O4. Central (e.g., PICC, tunneled, port)	☐
Z1. None of the Above	☑

PATIENT COORDINATION AND CARE PLAN

Plan of Care Orders

Orders and Treatment (Specify Frequency, Duration and Purpose of Visits): §484.60 (2.iv)

SN Frequency: 1/wk x 1wk, 2/wk x 1wk, 1/wk x 2wks

MSW Frequency:

PT Frequency: Evaluation and Treatment: 2/wk x 4wks

OT Frequency:

HHA Frequency:

ST Frequency

Dietitian Frequency:

MD SIGNATURE COVERS ALL PAGES

ADMIT PATIENT TO AVERY HOME HEALTH SERVICES. ALL DIAGNOSIS AND MEDICATIONS VERIFIED WITH THE PRIMARY CARE PHYSICIAN.

The patient is considered homebound because she has multiple active medical conditions, including polyneuropathy, muscle weakness, bilateral lower leg, memory impairment, and dependence on an enabling machine or device. These conditions significantly impair her mobility and ability to perform activities of daily living independently, resulting in difficulty walking, dressing, bathing, grooming, toileting, and doing errands alone. She is unable to walk independently without assistance or assistive devices and has transportation difficulties. Additionally, she experiences tremors, chronic pain, anxiety, depressive disorder, and seizures, which further complicate her functional status and safety.

Leaving the home requires a considerable and taxing effort due to her severe mobility impairments, muscle weakness, and neurological deficits such as foot drop and neuropathy. She also has a history of falls and requires maximal assistance, which increases the risk of injury if she attempts to leave the home unassisted. Any outings are limited to medically necessary appointments or essential activities, given her dependence on assistive devices and high fall risk.

These documented functional limitations and medical conditions justify the patient's homebound status and the medical necessity for skilled home health services to provide ongoing skilled nursing, physical and occupational therapy, and assistance with complex medical management and safety oversight.

Frequency and Duration:
HH SN frequency: 1 time a week for 1 week, 2 times a week for 1 week, 1 time a week for 2 weeks for skilled observation and assessment, medication reconciliation, pain assessment, wound monitoring and management, infection control measures, diet, home safety and disease process education.
HH PT frequency: 2 times a week for 4 weeks for evaluation and treatment for mobility, gait, transfer, establish therapeutic exercises, strengthening, home safety and fall prevention techniques training effective on week of 02/22/2026.

Goals:
Patient will improve lower extremity strength to facilitate safer transfers and ambulation within 4 weeks.
Patient will demonstrate safe use of assistive device for ambulation to reduce fall risk within 4 weeks.
Patient will increase independence with activities of daily living (dressing, bathing, grooming) within 4 weeks.
Patient will improve range of motion in affected limbs to reduce muscle stiffness within 4 weeks.
Patient will participate in cognitive exercises aimed at improving memory and attention within 4 weeks.

Patient Strengths

☐ **Motivated Learner**
☑ **Strong Support System**
☐ **Absence of Multiple Diagnosis**
☐ **Enhanced Socioeconomic Status**
Other:

Conclusions

☑ **Skilled Intervention Needed**
☑ **Skilled Instruction Needed**
☐ **No Skilled Service Needed**
Other:

Patient Summary / Clinical Narrative

Patient Alice Hylton is a 62-year-old female who was referred for home health care on February 13, 2026, and was admitted under the care of Dr. Julie Leinhauser. The start of care date was set for February 18, 2026. Alice's preferred language is English, and she has been assessed by Registered Nurse Kelvyn Garcia Alfonso. It is important to note that Alice was not discharged from an inpatient facility within the past 14 days, which indicates that her home health services are initiated without recent hospitalization.

Alice's homebound status has been established based on several criteria. She is considered homebound due to illness and has a medical condition that contraindicates leaving her home. This includes being chair-bound, unable to tolerate standing or walking for extended periods, and experiencing pain that impairs her mobility. She is also at risk for falls, relying on walls and furniture for support. Leaving home is exhausting for her, requiring extended rest periods afterward, and she needs assistance due to an unsteady gait and weakness from a progressive condition. The prognosis for Alice is guarded, with functional limitations in ambulation and endurance.

The primary reason for Alice's need for home health services is her diagnosis of G62.89, which refers to other specified polyneuropathies. Additionally, she has comorbidities including G56.03 (carpal tunnel syndrome in bilateral upper limbs), G89.29 (other chronic pain), M62.81 (generalized muscle weakness), M21.372 (left foot drop), and I10 (essential hypertension). These conditions contribute to her declining functional status and the need for consistent monitoring and assistance.

A comprehensive head-to-toe assessment indicates that Alice has a medium acuity level. Her vital signs show a temperature of 99°F, a pulse of 100 beats per minute, blood pressure at 152/92

mmHg, and a respiratory rate of 22 breaths per minute. Cardiopulmonary assessments reveal she experiences dyspnea when walking more than 20 feet or climbing stairs, reflecting her compromised respiratory function. Gastrointestinal assessments show no issues with bowel incontinence, and her integumentary system is stable with no signs of pressure ulcers.

Alice's psychosocial status indicates she experiences moderate depression, with a PHQ-2 score of 10. Although she feels anxious daily, she does not report feeling lonely or isolated. She requires assistance with decision-making, which occurs several times a week, indicating some cognitive impairment, although she is generally alert and oriented. Her cognitive function allows her to focus, shift attention, and comprehend task directions, although she occasionally experiences confusion.

In terms of activities of daily living (ADLs), Alice is entirely dependent on caregivers for grooming, dressing, and toileting, and while she can participate in bathing, she requires supervision throughout the process. Her functional ability is classified as dependent in self-care and indoor ambulation. Although she utilizes a motorized wheelchair, she has difficulties with feeding herself and requires assistance throughout meals.

Alice lives with non-agency caregivers who provide assistance with her daily needs. The social environment is supportive, yet challenges persist due to her medical conditions and restricted mobility. Her medication list includes five or more medications that require careful management to avoid adverse effects and interactions. Currently, she is taking opioids, classified as high-risk medications, for pain management, and she has received education on precautions related to these medications.

The medical necessity for skilled services in home health care is evident in Alice's comprehensive assessment. She requires frequent visits to monitor her vital signs, manage her medications, and provide support with her ADLs, particularly as she navigates her chronic pain and mobility limitations. It is recommended that she receive home health visits three to four times per week to ensure her safety, provide necessary care, and support her overall health and well-being.

Skilled Intervention

Assessment/Instruction/Performance:
Explained Home Health service to patient/family/caregiver(s). Explained consent to care, release of liability, and patient rights & responsibilities. Consent to care, a liability release, and consent to photograph were signed by the patient/responsible party. A comprehensive assessment on all body systems with OASIS elements evaluation was initiated. Reconciled facility discharge medications against routine home prescriptions without issue. Discussed advance directives, privacy policy, COVID-19 prevention, 911 protocols, basic home safety, and fall prevention principles with patient/family/caregiver(s). Discussed plan of care draft with patient and explained what to expect in the next visits and the remainder of the care episode. Also explained that the initial discussion on the management of other disease processes will be done on the next visit.

Skilled nursing interventions were provided including comprehensive assessment and monitoring of neurological status to identify any progression or complications. The patient received medication management to optimize pharmacologic treatment and minimize side effects. Additionally, education on skin care and pressure injury prevention was delivered to address sensory impairments and reduce the risk of skin breakdown. Coordination with the interdisciplinary team ensured that the patient's care plan was tailored to address functional limitations resulting from polyneuropathy.

Instructed patient to adhere and take pain medication before pain becomes severe to achieve pain control. Provided teachings on nonpharmacologic pain relief measures such as relaxation techniques and positioning. Instructed patient/PCG to notify SN/MD if patient is experiencing pain level greater than 7/10, unable to tolerate pain, and if pain is affecting ability to perform normal activities/ADLs.

Assessed pt's safety in environment, mobility and functional limitations to reduce falls or accidents every visit. Assessed the patient's ability to perform ADLs effectively and safely every visit. Instructed Patient/Caregiver on the importance of assessing conditions/risks that may contribute to occurrence of falls such as fatigue/ muscle weakness. Encouraged to perform exercises if tolerated for strengthening and maintenance of posture and flexibility. Established HEP and educate Patient/Caregiver on passive/active assistive ROM exercises and encouraged to perform exercises to maintain muscle strength and endurance.

Taught patient/caregiver on fall prevention techniques including changing position slowly, wearing proper footwear during ambulation, use of prescribed assistive device, importance of adequate lighting in the house, placing patient's items within arm's reach, remove clutter from patient's path as patient is fall risk and stress to patient to avoid attempting task that are too difficult for present level of functioning and ask for help when needed. Instructed the patient/caregiver to contact the agency to report any fall with or without minor injury and to call 911 for fall resulting in serious injury or causing severe pain or immobility.

Medication reconciliation was done during the visit. Patient/caregiver verbalized understanding in all teachings during the visit.

☑ **Response to Teaching/Interventions**

Verbalized Understanding	☑ **Patient (%):** 60%	☑ **Caregiver (%):** 75%
Return Demonstration	☑ **Patient (%):** 60%	☑ **Caregiver (%):** 75%
Require Further Teaching	☑ **Patient**	☑ **Caregiver**

Comments:
Patient verbalized 60% of understanding on the teachings taught during this visit. Patient also re-demonstrated 60% of infection control, pain management, home safety and fall prevention measures with the use of assistive device as per patient still demonstrates weakness, poor endurance, and ambulation difficulty. CG verbalized 75% of understanding and is available to assist patient for safety with ADLs/IADLs, medication, and diet compliance for the patient.

Conference With:

☑ **MD**	☑ **SN**	☑ **PT**	☐ **OT**
☐ **ST**	☐ **MSW**	☐ **HHA**	☐ **DON/DOPCS**
☐ **Clinical Coordinator**	☐ **Clinical Manager**		

Name:
Nariah Hinds, RN, CM
Regarding:
Patient SOC visit report and needs.
Physician Contacted Re:
Admission
Order Changes:
Continue to observe and assess all body systems, assess for functional status, knowledge of self-care management, and overall patient health condition and education on disease process and management, teachings for diet, further skilled teaching/instructions on action, schedule and side effects of medication, diet, fall prevention and pain management.
Plan for Next Visit:
Evaluate patient POC and supervision of others HH services and requests.
Next Physician Visit:
mm/dd/yyyy 📅

Patient and caregiver education and training to facilitate timely discharge: §484.60 (2.xiii)
Visit Narrative

Interventions/Instructions during visit:

☑ **Assess/Observe**	☐ **Injection Administration**	☐ **Catheter change**	☐ **IV Therapy**
☐ **Wound Care**	☑ **Teaching/Instructions**		

Discharge Plans

When goals are met.
Discharge when medical condition is stable and patient is no longer in need of skilled services.
Discharge to care of physician.
Discharge to caregiver.
Discharge patient to self-care.
Discharge when caregiver willing and able to manage all aspects of patient's care.
Discharge when goals met. Discharge when wound(s) healed.
Discharge when reliable caregiver available to assist with patient's medical needs.
Discharge when patient is independent in management of medical needs.
Able to return to an independent level of care.
Able to stay in residence with primary caregiver's assistance or community agency's support.
When patient knows when to notify physician.
Able to understand medication regime and care related to diagnoses.

☐ **Written notice of discharge provided to patient**

Discharge scheduled for:
mm/dd/yyyy 📅

Rehabilitation Potential

- ◯ Rehabilitation potential is poor
- ☑ Rehabilitation potential is fair
- ◯ Rehabilitation potential is good
- ◯ Rehabilitation potential is excellent

Others:

OTHER SUPPLEMENTALS

Endocrine

☑ WNL (Within Normal Limits)

Is patient diabetic?	◯ Yes	◯ No	
Insulin dependent?	◯ Yes	◯ No	How Long?
Is patient independently able to draw up correct dose of insulin?	◯ Yes	◯ No	◯ N/A
Is patient able to administer own insulin?	◯ Yes	◯ No	◯ N/A
Is patient taking oral hypoglycemic agent?	◯ Yes	◯ No	◯ N/A
Is patient independent with glucometer use?	◯ Yes	◯ No	◯ N/A
Is caregiver able to draw up and administer insulin?	◯ Yes	◯ No	◯ N/A
Is caregiver independent with glucometer use?	◯ Yes	◯ No	◯ N/A
Does patient or caregiver routinely perform inspection of the patient's lower extremities?	◯ Yes	◯ No	◯ N/A

Does patient have any of the following?

- ☐ Polyuria
- ☐ Polydipsia
- ☐ Polyphagia
- ☑ Neuropathy
- ☐ Radiculopathy
- ☐ Thyroid Problems

Comments

Past Medical History (Mark all that apply)

- ☐ CHF
- ☐ Cardiomyopathy
- ☐ Arrhythmia
- ☐ Chest Pain
- ☐ MI
- ☐ CAD
- ☑ HTN
- ☐ PVD
- ☐ Murmur
- ☐ Cancer: (Specify Type) — In remission? ◯ Yes ◯ No
- ☐ Osteoathritis/DJD: (Specify site affected)
- ☐ Joint Replacement: (Specify joint)
- ☐ Rheumatoid Arthritis
- ☑ Gait Problems
- ☐ Fractures
- ☑ Falls
- ☐ CVA
- ☐ TIA
- ☐ MS
- ☐ Hemiplegia
- ☐ Seizures
- ☐ Headaches
- ☐ Dizziness/Vertigo
- ☐ IBS
- ☐ Crohn's Disease
- ☐ Diverticulitis/Diverticulosis
- ☐ Constipation
- ☐ Diarrhea
- ☐ Fecal Incontinence
- ☐ Liver/Gallbladder Problems: (Specify)
- ☑ Depression
- ☑ Anxiety
- ☐ Dementia
- ☐ Substance Abuse: (Specify)
- ☐ Alzheimer's
- ☐ Mental Disorder: (Specify)
- ☐ Pressure Ulcer
- ☐ Stasis Ulcer
- ☐ Diabetic Ulcer
- ☐ Trauma Wound
- ☐ Other: (Specify)
- ☐ Chronic Kidney Disease
- ☐ Renal Failure
- ☐ Dialysis
- ☐ Anemia
- ☐ Abnormal Coagulation
- ☐ Blood Clots
- ☐ Diabetes
- ☐ Thyroid Problems
- ☐ COPD
- ☐ Asthma
- ☐ Emphysema
- ☐ Chronic Obstructive Bronchitis
- ☐ Chronic Obstructive Asthma
- ☐ Urinary Incontinence
- ☐ Urinary Retention
- ☐ BPH
- ☐ Recent/Frequent UTI
- ☐ Tuberculosis
- ☐ Hepatitis: (Specify)
- ☐ Infectious Disease: (Specify)
- ☐ Tobacco Dependence

Type	Amount	Length of Time Used

Past Surgical History

Most Recent Immunization

Pneumonia	◯ Yes	◯ No	☑ Unknown	Date mm/dd/yyyy
Flu	◯ Yes	◯ No	☑ Unknown	Date mm/dd/yyyy
Tetanus	◯ Yes	◯ No	☑ Unknown	Date mm/dd/yyyy
TB	◯ Yes	◯ No	☑ Unknown	Date

mm/dd/yyyy

TB Exposure
◯ Yes ◯ No ● Unknown **Date** mm/dd/yyyy

Hepatitis B
◯ Yes ◯ No ● Unknown **Date** mm/dd/yyyy

Additional Immunizations

Immunization Name
◯ Yes ◯ No ◯ Unknown **Date** mm/dd/yyyy

Immunization Name
◯ Yes ◯ No ◯ Unknown **Date** mm/dd/yyyy

Comments
Patient and caregiver unable to find the paper for immunization. Taught patient/caregiver the importance of up-to-date immunizations.

Vaccination Record

1st Dose COVID-19
Product Name/Manufacturer (Lot No.) **Date** mm/dd/yyyy **Healthcare Professional or Clinic Site**

2nd Dose COVID-19
Product Name/Manufacturer (Lot No.) **Date** mm/dd/yyyy **Healthcare Professional or Clinic Site**

Booster 1
Product Name/Manufacturer (Lot No.) **Date** mm/dd/yyyy **Healthcare Professional or Clinic Site**

Booster 2
Product Name/Manufacturer (Lot No.) **Date** mm/dd/yyyy **Healthcare Professional or Clinic Site**

Health Screening

Last Cholesterol Level mm/dd/yyyy
Last Mammogram mm/dd/yyyy

Does patient perform monthly self breast exam? ◯ Yes ● No
Last Pap Smear mm/dd/yyyy
Last PSA mm/dd/yyyy

Last Prostate Exam mm/dd/yyyy
Last Colonoscopy mm/dd/yyyy

Fire Assessment for Patient with Oxygen

☑ **Patient is not using oxygen**

Does patient have No Smoking signs posted?	◯ Yes	◯ No
• **Are the Patient and Caregiver educated regarding this?**	◯ Yes	◯ No
Does patient or anyone in the home smoke with oxygen in use?	◯ Yes	◯ No
• **Are the Patient and Caregiver educated regarding this?**	◯ Yes	◯ No
Are smoke detectors present and working properly?	◯ Yes	◯ No
• **Are the Patient and Caregiver educated regarding this?**	◯ Yes	◯ No
Does patient have a properly functioning fire extinguisher?	◯ Yes	◯ No
• **Are the Patient and Caregiver educated regarding this?**	◯ Yes	◯ No
Are oxygen cylinders stored properly?	◯ Yes	◯ No
• **Are the Patient and Caregiver educated regarding this?**	◯ Yes	◯ No
Are all electrical cords near oxygen intact and free for fraying?	◯ Yes	◯ No
• **Are the Patient and Caregiver educated regarding this?**	◯ Yes	◯ No
Does patient have an evacuation plan in case of fire?	◯ Yes	◯ No
• **Are the Patient and Caregiver educated regarding this?**	◯ Yes	◯ No
Are all cleaning fluids and aerosols stored away from oxygen, and not used when oxygen is in use?	◯ Yes	◯ No
• **Are the Patient and Caregiver educated regarding this?**	◯ Yes	◯ No
Does patient refrain from using petroleum products around oxygen?	◯ Yes	◯ No
• **Are the Patient and Caregiver educated regarding this?**	◯ Yes	◯ No
Does patient only used water-based body and lip moisturizers?	◯ Yes	◯ No
• **Are the Patient and Caregiver educated regarding this?**	◯ Yes	◯ No

Comments

CARE PLAN (PROBLEMS / GOALS / INTERVENTIONS)

Patient Care Plan / Goals / Interventions:

Problem: Vital Signs Assessment
Goals: Target Completion: 4 Week(s) Term: Long-term Status: Ongoing
Patient's vital signs will be within normal limits.
Interventions: Assigned Staff: SN

Check O2 saturation every visit for any shortness of breath on exertion, fatigue/weakness.
Progress Toward Goals:
No Progress Yet
Check VS every visit and report to PMD if patient vitals are not within the parameters.
Progress Toward Goals:
No Progress Yet

Problem: Cardiopulmonary
Goals: Target Completion: 4 Week(s) Term: Long-term Status: Ongoing
Demonstrate pursed-lip breathing exercises, deep breathing exercises, and energy-conserving measures.
Interventions: Assigned Staff: SN
Review patient/caregiver on methods to conserve energy, frequent rest periods, sitting instead of standing to perform tasks, and performing slower, purposeful movements which will limit fatigue.
Progress Toward Goals:
No Progress Yet
Instruct patient/caregiver on pursed lip breathing exercises, deep breathing exercises to alleviate dyspnea.
Progress Toward Goals:
No Progress Yet

Goals: Target Completion: 4 Week(s) Term: Long-term Status: Ongoing
Patient will be free from signs/symptoms of cardiac and respiratory distress during the certification period. Verbalize understanding of symptoms of cardiac complications and when to call 911.
Interventions: Assigned Staff: SN
Reinforce patient/caregiver on sodium limitations, managing stress, avoiding smoking/alcohol intake, and compliance with prescribed medications to control blood pressure.
Progress Toward Goals:
No Progress Yet
Encourage the patient/caregiver to provide a quiet environment, limiting stimuli to promote rest.
Progress Toward Goals:
No Progress Yet
Advise the patient to get up slowly to offset the feeling of dizziness.
Progress Toward Goals:
No Progress Yet
Instruct patient/caregiver to notify SN/MD for signs/symptoms of cardiac and respiratory distress like chest pain, cyanosis, dependent edema, jugular vein distention, palpitations, wheezing, increased respiratory rate, anxiety, headache, lightheadedness, and shortness of breath.
Progress Toward Goals:
No Progress Yet

Goals: Target Completion: 4 Week(s) Term: Long-term Status: Ongoing
Patient will be free from cardiopulmonary distress during the care episode. Patient's pulse and blood pressure will be within established parameters.
Interventions: Assigned Staff: SN
Reinforce patient/caregiver to check and record blood pressure, including heart rate.
Progress Toward Goals:
No Progress Yet
Assess cardiopulmonary status every visit.
Progress Toward Goals:
No Progress Yet

Problem: Other specified polyneuropathies
Goals: Target Completion: 4 Week(s) Term: Long-term Status: Ongoing
Improve mobility and prevent falls
Interventions: Assigned Staff: SN
Implement a prescribed physical therapy program focusing on strength and balance.
Progress Toward Goals:
No Progress Yet
Monitor the patient's response to mobility interventions and adjust as necessary.
Progress Toward Goals:
No Progress Yet
Involve family members in mobility exercises to promote safety and support.
Progress Toward Goals:
No Progress Yet

Goals: Target Completion: 4 Week(s) Term: Long-term Status: Ongoing
Manage pain and discomfort associated with polyneuropathy
Interventions: Assigned Staff: SN
Coordinate with the physician for medication management and adjustments as needed.
Progress Toward Goals:
No Progress Yet
Provide comfort measures such as hot/cold therapy and relaxation techniques.
Progress Toward Goals:
No Progress Yet
Encourage family members to assist in pain management strategies and emotional support.
Progress Toward Goals:
No Progress Yet

Goals: Target Completion: 4 Week(s) Term: Long-term Status: Ongoing
Enhance daily living activities and independence
Interventions: Assigned Staff: SN
Provide recommendations for assistive devices to facilitate daily living activities.
Progress Toward Goals:
No Progress Yet
Monitor and support patient progress in completing ADLs.
Progress Toward Goals:
No Progress Yet
Involve family members in training for assisting with daily living activities.
Progress Toward Goals:
No Progress Yet

Goals: Target Completion: 4 Week(s) Term: Long-term Status: Ongoing
Support emotional well-being and coping strategies
Interventions: Assigned Staff: SN
Assess the patient's mental health status and provide appropriate referrals if needed.
Progress Toward Goals:
No Progress Yet
Provide education on stress management techniques and mindfulness practices.
Progress Toward Goals:
No Progress Yet

Goals: Target Completion: 4 Week(s) Term: Long-term Status: Ongoing
Nutritional support to manage overall health
Interventions: Assigned Staff: SN
Educate the patient and family on healthy eating habits and meal preparation techniques.
Progress Toward Goals:
No Progress Yet
Monitor the patient's nutritional intake and weight regularly.
Progress Toward Goals:
No Progress Yet
Encourage family involvement in meal planning and preparation to support adherence.
Progress Toward Goals:
No Progress Yet

Problem: Carpal tunnel syndrome affecting bilateral upper limbs.

Goals: Target Completion: 4 Week(s) Term: Long-term Status: Ongoing

Reduce pain and discomfort in the upper limbs.

Interventions: Assigned Staff: SN

Encourage the use of ice packs to reduce swelling and pain.

Progress Toward Goals:

No Progress Yet

Monitor and assess pain levels regularly to adjust care plan as needed.

Progress Toward Goals:

No Progress Yet

Educate family members on signs of increased pain or complications.

Progress Toward Goals:

No Progress Yet

Goals: Target Completion: 4 Week(s) Term: Long-term Status: Ongoing

Improve wrist and hand function.

Interventions: Assigned Staff: SN

Provide assistive devices to aid in daily activities and reduce strain on the hands.

Progress Toward Goals:

No Progress Yet

Conduct regular assessments of hand function and modify exercises accordingly.

Progress Toward Goals:

No Progress Yet

Engage family members in supportive roles during exercise sessions.

Progress Toward Goals:

No Progress Yet

Goals: Target Completion: 4 Week(s) Term: Long-term Status: Ongoing

Enhance overall quality of life.

Interventions: Assigned Staff: SN

Encourage family involvement in care routines to promote emotional support.

Progress Toward Goals:

No Progress Yet

Monitor emotional well-being and refer to mental health services if necessary.

Progress Toward Goals:

No Progress Yet

Educate the patient and family on the importance of regular follow-ups.

Progress Toward Goals:

No Progress Yet

Problem: Other chronic pain

Goals: Target Completion: 4 Week(s) Term: Long-term Status: Ongoing

Reduce pain levels to improve daily functioning.

Interventions: Assigned Staff: SN

Administer prescribed pain medications and monitor their effectiveness.

Progress Toward Goals:

No Progress Yet

Educate the patient and family about pain management techniques.

Progress Toward Goals:

No Progress Yet

Implement a schedule for regular pain assessments to track progress.

Progress Toward Goals:

No Progress Yet

Goals: Target Completion: 4 Week(s) Term: Long-term Status: Ongoing

Enhance the patient's ability to perform activities of daily living (ADLs).

Interventions: Assigned Staff: SN

Involve family members in training for ADL support to encourage independence.

Progress Toward Goals:

No Progress Yet

Evaluate home environment for safety and accessibility adjustments.

Progress Toward Goals:

No Progress Yet

Provide ongoing education to the family regarding the impact of chronic pain on daily activities.

Progress Toward Goals:

No Progress Yet

Goals: Target Completion: 4 Week(s) Term: Long-term Status: Ongoing

Promote a holistic approach to pain management.

Interventions: Assigned Staff: SN

Assess and recommend alternative therapies (e.g., acupuncture) to complement medical management.

Progress Toward Goals:

No Progress Yet

Involve family in pain management strategies to ensure support and understanding.

Progress Toward Goals:

No Progress Yet

Problem: Muscle weakness (generalized)

Goals: Target Completion: 4 Week(s) Term: Long-term Status: Ongoing

Improve muscle strength and mobility

Interventions: Assigned Staff: SN

Educate the family on how to assist with exercises safely at home.

Progress Toward Goals:

No Progress Yet

Monitor and record progress in muscle strength and adjust the plan accordingly.

Progress Toward Goals:

No Progress Yet

Incorporate assistive devices as needed to enhance mobility and safety.

Progress Toward Goals:

No Progress Yet

Goals: Target Completion: 4 Week(s) Term: Long-term Status: Ongoing

Enhance nutritional intake to support muscle health

Interventions: Assigned Staff: SN

Educate family members on meal preparation and healthy food choices.

Progress Toward Goals:

No Progress Yet

Monitor weight and nutritional status regularly to ensure adequate intake.

Progress Toward Goals:

No Progress Yet

Provide nutritional supplements if necessary to meet dietary goals.

Progress Toward Goals:

No Progress Yet

Goals: Target Completion: 4 Week(s) Term: Long-term Status: Ongoing

Support the patient’s emotional well-being during recovery

Interventions: Assigned Staff: SN

Encourage participation in social activities that promote engagement and reduce isolation.

Progress Toward Goals:
No Progress Yet
Assess for any signs of depression or mental health concerns and refer as needed.
Progress Toward Goals:
No Progress Yet
Educate family on the importance of emotional support and active listening.
Progress Toward Goals:
No Progress Yet

Problem: Foot drop in left foot
Goals: Target Completion: 4 Week(s) Term: Long-term Status: Ongoing
Improve mobility and gait stability
Interventions: Assigned Staff: SN
Assess the need for orthotic devices and assist the family in obtaining necessary equipment.
Progress Toward Goals:
No Progress Yet
Monitor patient's progress and adjust the exercise plan as necessary.
Progress Toward Goals:
No Progress Yet
Provide education on fall prevention strategies in the home environment.
Progress Toward Goals:
No Progress Yet

Goals: Target Completion: 4 Week(s) Term: Long-term Status: Ongoing
Enhance independence in daily activities
Interventions: Assigned Staff: SN
Implement adaptive techniques for daily living activities to accommodate foot drop.
Progress Toward Goals:
No Progress Yet
Teach family members how to assist the patient with mobility-related tasks safely.
Progress Toward Goals:
No Progress Yet
Create a safe and accessible living space by removing potential hazards.
Progress Toward Goals:
No Progress Yet

Problem: Essential (primary) hypertension
Goals: Target Completion: 4 Week(s) Term: Long-term Status: Ongoing
Reduce blood pressure to within normal range (<120/80 mmHg)
Interventions: Assigned Staff: SN
Monitor blood pressure weekly and maintain a log for review.
Progress Toward Goals:
No Progress Yet
Educate the patient and family on the importance of medication adherence.
Progress Toward Goals:
No Progress Yet
Assess dietary habits and recommend dietary changes to lower sodium intake.
Progress Toward Goals:
No Progress Yet

Goals: Target Completion: 4 Week(s) Term: Long-term Status: Ongoing
Improve understanding of hypertension management among the patient and family.
Interventions: Assigned Staff: SN
Provide educational materials on hypertension and its management.
Progress Toward Goals:
No Progress Yet
Conduct a family meeting to discuss the care plan and answer questions.
Progress Toward Goals:
No Progress Yet
Teach the patient and family how to recognize symptoms of high blood pressure.
Progress Toward Goals:
No Progress Yet

Problem: Fall Risk
Goals: Target Completion: 4 Week(s) Term: Long-term Status: Ongoing
Reduce the patient's risk of falls in the home environment.
Interventions: Assigned Staff: SN
Educate the patient and family on fall prevention strategies.
Progress Toward Goals:
No Progress Yet
Review and adjust medications that may contribute to dizziness or drowsiness.
Progress Toward Goals:
No Progress Yet
Provide assistive devices (e.g., walkers, grab bars) to enhance mobility and safety.
Progress Toward Goals:
No Progress Yet

Goals: Target Completion: 4 Week(s) Term: Long-term Status: Ongoing
Increase patient and family awareness of the importance of fall prevention.
Interventions: Assigned Staff: SN
Provide educational materials on fall prevention for the family.
Progress Toward Goals:
No Progress Yet
Involve family members in regular safety checks of the home environment.
Progress Toward Goals:
No Progress Yet
Set up a fall alert system (e.g., emergency pendant) for the patient.
Progress Toward Goals:
No Progress Yet

Problem: Pressure Ulcer Prevention
Goals: Target Completion: 4 Week(s) Term: Long-term Status: Ongoing
Verbalize understanding of the importance of pressure-relieving measures.
Interventions: Assigned Staff: SN
Instruct patient/caregiver on pressure-relieving measures such as repositioning every 2 hours, float heels, pad bony prominences and methods to reduce friction and shear.
Progress Toward Goals:
No Progress Yet

Goals: Target Completion: 4 Week(s) Term: Long-term Status: Ongoing
Interventions: Assigned Staff: SN
Instruct patient/caregiver to inspect skin daily and notify SN/MD for redness or open sores.
Progress Toward Goals:
No Progress Yet
Assess skin for breakdown due to risk of pressure ulcer development.
Progress Toward Goals:
No Progress Yet

Problem: Rehospitalization Risk and Medication Management

Goals: Target Completion: 4 Week(s) Term: Long-term Status: Ongoing
Patient will be free from side effects and adverse medication reactions during the entire episode. Patient/Caregiver will demonstrate medication safety and compliance. Patients will have no re-hospitalizations.
Interventions: Assigned Staff: SN
Identify Medication Interactions and evaluate patient/family/caregiver(s) understanding of medication compliance.
Progress Toward Goals:
No Progress Yet

Ongoing Care Plans within OASIS date (02/18/2026):

Assigned Staff: Garcia Alfonso, Kelvyn RN

Electronically Signed by Garcia Alfonso, Kelvyn RN 02/18/2026